

7 June 2023

Lucy Letby - Cross-examination Day 8: Babies M & N

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MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

Wednesday, 7 June 2023
(10.30 am)

(In the presence of the jury)

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

MR JUSTICE GOSS: Mr Johnson.

MR JOHNSON: On Monday, before the midday adjournment, we dealt with pretty much the whole of three cases: [Baby K], [Baby F] and [Baby L]. Is there anything that you said about any of those three children that you, on reflection, wish to amend?

LETBY: No.

JOHNSON: And so far as the wider case is concerned, in other words questions that I have been asking you since we started this exercise, is there anything that you wish to amend?

LETBY: No.

JOHNSON: Do you give that some thought at the end of each day, that issue?

LETBY: Yes.

JOHNSON: So you -- it's not a criticism at all, but you reflect on the evidence you have given?

LETBY: As much as I can remember, yes.

JOHNSON: Yes, and you think about whether or not there's something that you have said that, with hindsight, you might either wish to change or re-emphasise or add to or take away from?

LETBY: Yes.

JOHNSON: Yes. And each time I have asked you that question, you have confirmed that, in effect, there is nothing you wish to change?

LETBY: Yes.

JOHNSON: Thank you. Now, if we can move on to the case relating to [Baby M], please, who is named in count 16 on the indictment. I'd like to start, as I have done from time to time, with your defence statement. All right? The jury now have a typed copy of the statement.

The part I'd like to ask you about starts at paragraph 137, which in the jury's copy is page 17.

I've dealt with parts of this already because, as we see from the document, this section of it concerns both brothers, [Baby L] and [Baby M]. All right?

LETBY: Yes.

JOHNSON: So on Monday we dealt with the parts that refer simply to [Baby L] --

LETBY: Yes.

JOHNSON: -- and we will turn now to [Baby M].

At paragraph 137 you say that after the twins were born on 8 April, they were put into nursery 1, that you and Amy Davies took them there, and you looked after them on their first day, you had looked after [Baby L] and Amy looked after [Baby M].

LETBY: Yes.

JOHNSON: And I asked you some questions about that on Monday so far as [Baby L] was concerned.

LETBY: Yes.

JOHNSON: At 138 you say:

"[Baby M] was not put in a proper space. He was slotted into a space by a wall with a portable Masimo monitor. He was fitted in this way because nursery 1 was full."

You say.

LETBY: Yes.

JOHNSON: You then move on to say that:

"We put a crash call out. [Nurse B] came and tried to turn the incubator around to get [Baby M] on to a proper monitor."

LETBY: Sorry, where are we now?

JOHNSON: 146, I beg your pardon. We've moved on. 139 to 145 relate to [Baby L]. My mistake.

You just check for yourself that I've not missed anything out between 139 and 146.

(Pause)

LETBY: It just refers to -- about [Baby M], about the Neopuff on the wall at 144 and 143.

JOHNSON: Right, okay. You're quite right. 143:

"[Baby M] collapsed as Mary and I were making up fluids for [Baby L]. His alarm went off so we went over to him and he was apnoeic. When I look back, I'm not sure if he wasn't breathing or whether he'd had a desaturation."

Is that right?

LETBY: Yes.

JOHNSON: Then you say:

"I used the Neopuff that was on the wall, but because of how [Baby M] had been fitted into the nursery it wouldn't reach the cot and the mask pulled off. Someone brought me a new Neopuff mask."

LETBY: Yes.

JOHNSON: So 145:

"We asked Belinda Simcock and Ashleigh Hudson to do the bag for [Baby L] while we dealt with [Baby M]."

So the bag in that context of course is the more concentrated dextrose that Mary Griffith had started to put together?

LETBY: Yes.

JOHNSON: And then:

"We put a crash call out [this is the bit where I had jumped to]. [Nurse B] came and tried to turn the incubator round to get

[Baby M] on to a proper monitor. Dr Jayaram then came in and probably other doctors. Eventually, we moved [Baby M] to the intensive care side of the nursery because we needed a ventilator."

You then say at 147:

"I didn't notice any change in skin colour in [Baby M], although I have not dealt with many sick Asian babies, so I may not have been accustomed to the changes in skin colour. However, no one mentioned any changes in skin colour at the time."

You then say at 148 you took care of [Baby M] once he came off the ventilator:

"He'd been given medication prior to his collapse. I can't recall if it was me or Mary Griffith who did this."

And you then turn back in the document to [Baby L] at 150 and 151.

LETBY: Yes.

JOHNSON: Then 152:

"Sometimes when we do a resuscitation we write notes on whatever is available. This would sometimes be a paper towel. That is happened [it maybe should read 'that is what happened'] with [Baby M]. I wrote notes on the paper towel which then remained in my pocket and ended up at home with me."

I just want to understand what you're saying.

I have asked you some questions about this and I will return to it, but I just want to understand what it is you're actually saying in the document there.

Are you saying in the document that once you had written notes on the paper towel it went into your pocket and that is where it remained until it got home?

LETBY: I'm saying that I wrote notes on the paper towel, yes, and at the end of the day it came home with me in my pocket.

JOHNSON: And where had it been in between?

LETBY: Well, it would have been used during the shift to write up notes.

JOHNSON: All right. We'll come back to that.

At 153:

"A number of my diary entries were written retrospectively, for example 'Twins resus'. Matters like this were written after I'd been removed from the unit. I was documenting matters that I believed might be significant. LD stands for long day."

And that's how you deal with the case of [Baby M] in this document.

LETBY: Yes.

JOHNSON: Was there anything that was particularly memorable about [Baby M] so far as you were concerned?

LETBY: The delivery of the twins stood out in my mind because that was the first delivery I'd been to where I was the allocated nurse for a baby by myself, so yes, as a pair of twins, yes, they stood out in my mind, yes.

JOHNSON: I want to see whether you agree with a number of propositions. All right? First, that [Baby M] was born in good condition?

LETBY: Yes.

JOHNSON: That within a few hours his observations were being taken every 2 hours and he was breathing for himself in air?

LETBY: Yes, if that's -- I obviously can't see the chart now, but yes.

JOHNSON: Okay. Well, if you want to look at the chart, please don't let me rush you beyond it if you want to. They're all behind divider 16 in that bundle that you have in front of you.

I'm doing my best to reflect accurately the evidence that's been given, but it may be that I from time to time I can make a mistake. So if you want to check anything, you say so. All right?

LETBY: Okay.

JOHNSON: But do you agree with those propositions?

LETBY: Could you repeat the last one?

JOHNSON: Yes: obs every 2 hours, breathing for himself in air; that's within a few hours of his birth.

LETBY: Yes.

JOHNSON: He was not an intensive care baby, was he?

LETBY: Um, no, I don't believe he was at this time, no.

JOHNSON: No. Is it your case that the staffing levels generally caused or contributed to [Baby M]'s collapse?

LETBY: I don't know what caused [Baby M]'s collapse, but the unit was very stretched that day with staffing, yes.

JOHNSON: Yes, but you will understand, I'm sure, that it has been suggested on your behalf from time to time, and you gave evidence in answer to questions from your own counsel from time to time, that may have been taken as suggesting that there is a connection between staffing levels and collapses. All right? And I hope you understand that what I'm trying to do is to look at each case individually and see whether in fact there is a connection on your evidence.

LETBY: Okay.

JOHNSON: So putting to one side for a second the fact that you have told us that you don't know what the cause of [Baby M]'s collapse was, are you suggesting that there is a positive association between staffing levels and his collapse?

LETBY: I think that's a potential, yes.

JOHNSON: Well, a potential -- what is the potential connection?

LETBY: Because I don't know exactly what happened to [Baby M], but I know we were very stretched that day in providing all of the needs on the babies on that unit to make sure that they were all cared for. It was very stretched for every child --

JOHNSON: But are you --

LETBY: -- and [Baby M] wasn't in a correct space either.

JOHNSON: Are you pointing to a positive event which, had staffing levels been different, would have been dealt with differently?

LETBY: Potentially, yes, if [Baby M] had been in a proper space and we hadn't been as stretched with staff, maybe things would have been acted on quicker, I don't know.

JOHNSON: What difference would the space have made?

LETBY: He wasn't on a correct monitor. He wasn't -- he was just in a corner space, which isn't ideal. We don't usually have babies in that space.

JOHNSON: Well, we'll come to the population distribution in a moment. But you are postulating the possibility, are you, that had he been on a Philips monitor there may have been an earlier alarm than in fact there was?

LETBY: Potentially, yes.

JOHNSON: Right. That doesn't have anything to do with staffing levels, though, does it?

LETBY: No.

JOHNSON: That's to do with the monitor.

LETBY: Yes.

JOHNSON: So at the risk of repeating myself, just going back to the staffing levels, what are you suggesting is the connection?

LETBY: Again, I don't know what happened to [Baby M], but as nursing and medical staff we were very stretched that day. Um... Sorry, can you -- what was I saying, sorry?

JOHNSON: I'm looking to giving you an opportunity, really, of telling the jury what you are suggesting is the connection between staffing levels and [Baby M]'s collapse.

LETBY: As I say, I don't know what happened to [Baby M], but the unit was very busy that day, staffing wasn't at a great level, the doctors were very busy, as were the nurses.

Potentially, babies on the unit at that time didn't have such close monitoring as maybe they would have done if there'd been more staff or less babies.

JOHNSON: Are you suggesting that anybody's mistake contributed in any way to [Baby M]'s collapse?

LETBY: I don't know what caused [Baby M]'s collapse.

JOHNSON: So is the answer no?

LETBY: Yes.

JOHNSON: Are you suggesting that medical or nursing competence made a contribution?

LETBY: Again, that's hard to answer when I don't know exactly what happened to [Baby M]. I know he had had a few issues earlier on in the day. Maybe that could have been dealt with differently, I don't know.

JOHNSON: Right. Well, when we get to them, if you are suggesting that things should have been dealt with differently, will you say so, please --

LETBY: Yes.

JOHNSON: -- and tell us what the difference could or should have been; yes? Thank you.

Now, I'm sure you will remember that Dr Jayaram told us of discolouration that he saw on [Baby M] at the time of his collapse.

LETBY: Yes.

JOHNSON: I want to put to you his description and ask whether you disagree with what he says; okay?

LETBY: Okay.

JOHNSON: What he said was this, it was on 22 February if anybody wants to check:

"Because [Baby M] was darker skinned, it was more obvious. He was generally pale, which one would expect in a cardiorespiratory arrest. There were patches of very bright pink or certainly more obviously pink because it was on a darker-skinned background on his torso that flitted around. These sort of pink blotches for want of a description, better -- patches. It would appear and disappear and then other ones would appear and disappear. I noticed them on his abdomen because that's the most obvious bit of his body that you could see because the rest of his body had people covering his chest, doing CPR. I noted them sort of when I got there during the start of the resuscitation. I can't comment on how long they continued to be there. Once circulation was restored and his heart rate came above 100, they vanished."

And he said he'd never seen that type of a discolouration before [Baby A]. Okay?

LETBY: Yes.

JOHNSON: Do you agree with his description?

LETBY: I did not see anything like that, no.

JOHNSON: Did the lighting in nursery 1 make it difficult to see?

LETBY: No.

JOHNSON: Because you've told us, haven't you, that the lighting in nursery 1 is bright?

LETBY: Yes.

JOHNSON: This was a daytime collapse.

LETBY: Yes.

JOHNSON: And so the lighting would have been on full, wouldn't it?

LETBY: Um, it wouldn't have been on full, no, because no nursery light is on full unless you're actually doing a procedure. But yes, it would have had a fair level of lighting, yes.

JOHNSON: Is resuscitation a procedure?

LETBY: Yes, the lights would have been turned up then, yes.

Sorry, I thought you meant during the day.

JOHNSON: I'm talking about when Dr Jayaram speaks of having seen what I've just recounted to you.

LETBY: Yes.

JOHNSON: Do you remember what you said to the police about the lighting in this particular case?

LETBY: Yes, that it was poor where -- the space that [Baby M] was.

JOHNSON: Mm. If we just go to your interview, it's in bundle 2 of the interviews and it's the [Initials of Baby M] section, which is about halfway through. [Initials of Baby M] for [Baby M]. Go to page 4, please, [Initials of Baby M] 4. It's the first of the [Baby M] interviews. Do you have an independent memory of saying this or have you been reading your interviews to see what you said before I ask you questions about it?

LETBY: No, I have a direct memory of that event and needing to move [Baby M] into a different space.

JOHNSON: Right at the bottom of page [Initials of Baby M] 4, you're asked a question:

"Question: Do you know why he was desaturated?"

"Answer: No.

"Question: So when you attended to him there was nothing obvious that had caused this desaturation?"

"Answer: Not that I remembered, no. And then that officer says - a different officer says:

"Any more questions?"

And you give a reply, don't you? Would you just read your reply, please.

LETBY: "I do remember that his colour was a little bit harder to assess with him being an Asian baby and also he was in a corner space where there's poorer lighting."

JOHNSON: So were you suggesting to the police in this interview that the poor quality of the lighting during the daytime in nursery 1 may have been a reason why you didn't see a change in colouration?

LETBY: Yes. So at the time that [Baby M] collapsed, the nursery was not in full brightness, so when I went over to [Baby M] he was in a darker corner of the nursery.

JOHNSON: But a much lighter corner than nursery 2?

LETBY: Yes.

JOHNSON: You know what I'm going to say, don't you?

LETBY: Yes.

JOHNSON: Yes. What am I going to say?

LETBY: That I was able to see [Baby I].

JOHNSON: Yes, in the dark.

LETBY: Yes. Well, not in complete darkness, but...

JOHNSON: So this time, you're saying that although a nursery was in daytime with the lighting on, the lighting wasn't good enough for you to have been able to see a change in colour?

LETBY: No, it was harder for me to assess because I was not used to caring for Asian babies, so the colour change, if any, was

very difficult for me to see. I did not notice any colour change.

JOHNSON: I want to turn to the population layout, please, at tile 83, Mr Murphy. We can put the interviews away now, please.

Here we can see that there were four children in nursery 1.

LETBY: Yes.

JOHNSON: How many spaces are there in nursery 1?

LETBY: Four.

JOHNSON: Why was it necessary to put [Baby M] in a corner space if there are four spaces and four children?

LETBY: The procedure is because we are an emergency unit, there always has to be one incubator free --

JOHNSON: I see.

LETBY: -- because obviously we don't know what might come through from delivery ward. So there is always a space set up that's available for any admissions.

JOHNSON: Okay. So is this a situation where, as you have told us, because [Baby M] was not an intensive care baby, he didn't actually need to be in nursery 1 at all?

LETBY: Yes, there was just no room on the outside nurseries, yes.

JOHNSON: Yes, we can see the layout there, can't we?

LETBY: Yes.

JOHNSON: We see that you were responsible for GT and TSB?

LETBY: Yes.

JOHNSON: And that your friend Mary Griffith was responsible for [Babies L & M]?

LETBY: Yes.

JOHNSON: As you have told us, four children in nursery 2, three of whom being looked after by Ashleigh Hudson, one by [Nurse B].

LETBY: Yes.

JOHNSON: [Nurse B] had a second child in nursery 4, who was in there with three others, being looked after by Angela McShane, and then Belinda Williamson had the three children in nursery 3.

LETBY: Yes.

JOHNSON: Was the capacity at the time 16?

LETBY: Yes.

JOHNSON: So making the allowance for the emergency provision of a single ITU space, you were running, in effect, at capacity?

LETBY: Yes.

JOHNSON: Do you remember describing Mary as quite a junior member of staff, Mary Griffith?

LETBY: Yes.

JOHNSON: How many years' experience did Mary have?

LETBY: I can't answer that, but Mary only did band 5 roles intermittently. She was predominantly a nursery nurse on the unit and would sometimes go up to a band 5 on days when we were short-staffed, so she wasn't predominantly a band 5 at all times.

JOHNSON: Moving on to tile 88, please, do you remember Dr Ukoh saw [Baby M] at 10.25 on the ward round?

LETBY: Yes.

JOHNSON: Did you have anything to do with [Baby M] at this stage?

LETBY: I couldn't answer that, I don't know.

JOHNSON: All right. Let's look at what you were doing at the beginning of the shift, if we can. Keeping that on the screen, but if you just put away those interviews, please --

LETBY: That's not the interviews.

JOHNSON: That's not the interviews? What's that?

MR JUSTICE GOSS: That's the second jury bundle.

MR JOHNSON: It's the first one actually, but do you want to just put that one away as well. If we could get out the black neonatal review, please, so it's the second neonatal review

bundle, and turn to the [Surname of Babies L & M] section of that. It's the third document in there.

On the front of it, it says:

"Events 17 and 18: [Baby L] and [Baby M]."

Do you have that?

LETBY: Yes.

JOHNSON: On the second page you have the layout we've just seen on the screen.

LETBY: Yes.

JOHNSON: And then on the third page, we have what in effect is a summary of other activity with other children going on at about this time.

LETBY: Yes.

JOHNSON: If we go to page 4 to deal with the time at which Dr Ukoh was looking at [Baby M], the notes of which we have on the screen, we see blue typescript showing what you were up to at about this stage.

LETBY: Yes.

JOHNSON: Just turn back a page to page 3, please, which is the first page of the table in effect. We see you doing observations for GT, don't we, at 8.30?

LETBY: Yes.

JOHNSON: Some medications at 9.05 to 9.11 --

LETBY: Yes.

JOHNSON: -- between lines 29 and 32?

LETBY: Yes.

JOHNSON: Also checking some medications for AW who was in nursery 2, being looked after by Ashleigh Hudson?

LETBY: Yes.

JOHNSON: Then going to page 4, lines 44, 49, 58 and 66, are you doing observations, feeding and making a note relating to the child TSB?

LETBY: Yes.

JOHNSON: Who was one of your two children in nursery 1?

LETBY: Yes.

JOHNSON: The child GT was, if this document is correct, which we believe it to be, very low maintenance; would you agree?

LETBY: No.

JOHNSON: Let's look at how GT was occupying your time then. At line 22, so back to page 3, do we see you feeding GT --

LETBY: Yes.

JOHNSON: -- at 9 am? That's the first thing you did on the shift that's been recorded by you?

LETBY: Yes.

JOHNSON: Lines 29 to 32, the medications that we spoke of earlier?

LETBY: Yes.

JOHNSON: At about the same time. The next entry for GT is at midday, isn't it?

LETBY: Yes.

JOHNSON: You see, that's why I'm suggesting GT is low maintenance so far as you are concerned.

LETBY: I disagree because the baby at that time was in nursery 1, it had cannulation issues, she was re-cannulated during that shift. She would sometimes need suction. She had been transferred into nursery 1 because she was unwell. That's why she'd been moved into nursery 1. She was initially in the special care nurseries.

JOHNSON: You've got an extraordinary memory for the detail of this child, haven't you?

LETBY: I was because she was a very complex baby that was on the unit a very long time, yes.

JOHNSON: But no memory of [Baby D] --

LETBY: No.

JOHNSON: -- who died in the same nursery that you were working in?

LETBY: Yes.

JOHNSON: Do you agree, just if we could put the doctor's note from 10.30 back up then, 10.25 I should say, that this examination of [Baby M] was unremarkable?

LETBY: Yes. I think he assessed [Babies L & M] after the two other babies because they were more special care than the other two, yes.

JOHNSON: You remember that as well?

LETBY: Yes, because I remember the baby GT needed a cannula replacing and I do recall that with the doctor, and I remember the doctor coming to see TSB, who I was concerned about at the time.

JOHNSON: I'm not asking you to say it, but do you remember TSB's name as well?

LETBY: Yes.

JOHNSON: From 7 years ago?

LETBY: Yes.

JOHNSON: If we go to tile 98, please. We see there at 12.30 pm a note made by [Nurse B]. Can you see it?

LETBY: Yes.

JOHNSON: With her initials at the bottom, referring to a nasogastric tube feed and the fact that she obtained an acidic aspirate.

LETBY: Yes.

JOHNSON: She said that [Baby M] was fine.

LETBY: Yes.

JOHNSON: Do you agree or disagree with her?

LETBY: Well, bile isn't a normal finding.

JOHNSON: Where is bile?

LETBY: So [Nurse B] has put:

"Partially digested stomach acid/bile, 1.5. Bile-stained."

JOHNSON: That's in Mary's entry at 2.30.

LETBY: Oh sorry, is it not a continuation?

JOHNSON: No.

LETBY: Sorry. Then yes.

JOHNSON: Well, it's an easy mistake to make, but we'll move on to that because I was going to deal with that anyway.

There we see, as you have already pointed out, Mary Griffith's recording of a 1.5ml bile-stained aspirate.

LETBY: Yes, apologies. I was reading it on from [Nurse B]'s writing.

JOHNSON: It's all right, there's no need to apologise. But you will remember, I'm sure, that Dr Ukoh gave evidence that it was at that stage -- or Mary said that Dr Ukoh put [Baby M] nil by mouth at that stage, and we can see that fact recorded, or at least the nil by mouth element of it, recorded just above, can't we?

LETBY: Yes.

JOHNSON: And the nasogastric tube was put on to free drainage.

LETBY: Okay.

JOHNSON: That was the evidence. What's the point of putting the NGT on free drainage?

LETBY: Bile is not a normal finding and that should not be in the stomach, so the tube would go on to free drainage to ensure that anything that is in the stomach comes straight out and doesn't sit within the stomach and that we can monitor what's coming out.

JOHNSON: At 15.30, so we're moving on now, please, to tile 127, a dextrose bag was started for [Baby M] by you and Mary Griffith.

LETBY: Yes.

JOHNSON: And there is nothing to suggest, is there, that there was any insulin in that bag?

LETBY: No.

JOHNSON: No. You see, I'm asking you this question because I suggested to you yesterday that [Baby L], who was receiving insulin from a dextrose bag at this point, was targeted specifically. Okay?

LETBY: Okay.

JOHNSON: There's nothing in any of [Baby M]'s findings to support the suggestion that he was receiving insulin, is there?

LETBY: No.

JOHNSON: No. At 15.30 we are now about half an hour or so before [Baby M] collapsed.

LETBY: Yes.

JOHNSON: Looking at Dr Ukoh's -- is that Dr Ukoh's prescription, do you know, or writing? It looks like his --

LETBY: I couldn't comment, sorry.

JOHNSON: I think he said it was, but it may not matter. Do we see your signature above that of Mary Griffith?

LETBY: Yes.

JOHNSON: The rate and the time of the infusion being started, is that your writing as well?

LETBY: Um, yes.

JOHNSON: Can we rely on that time?

LETBY: Yes.

JOHNSON: Can you remember what Mary was doing at about this time though?

LETBY: No.

JOHNSON: Was she taking blood for the lab for [Baby L]?

LETBY: I can't say without looking.

JOHNSON: Well, we've all seen the paperwork relating to the blood test for [Baby L], haven't we?

LETBY: Yes.

JOHNSON: Do you remember the evidence that she gave that the podding of the sample was interrupted by the emergency with [Baby M]?

LETBY: Yes.

JOHNSON: How long in the normal course of events would it take between the taking of a sample and the podding of a sample?

LETBY: That would be dependent on staffing on the unit because it would require somebody to leave the unit to take the blood to be podded. If you had normal staffing, staff that could leave, it would be 5 or 10 minutes.

JOHNSON: Yes. There was another job going on, of course, at this same time as well, wasn't there; do you remember?

LETBY: Are you talking about the 12.5%?

JOHNSON: I am.

LETBY: Yes.

JOHNSON: You do remember?

LETBY: Sorry?

JOHNSON: And you do remember?

LETBY: Making up the 12.5%, yes.

JOHNSON: Yes. We've seen a video of the way this is done, haven't we?

LETBY: Yes.

JOHNSON: And one person, one of the two nurses involved, is the person who actually does the mechanical side of it?

LETBY: Yes.

JOHNSON: Is that right? That was Mary's function?

LETBY: Yes.

JOHNSON: What were you actually doing physically? Practically speaking, what did your role involve?

LETBY: So generally, the role would involve opening up any equipment that Mary would need and dropping it on to the sterile field and then watching everything that Mary does to ensure that the calculations are correct.

JOHNSON: And what does Mary do that actually has to be checked?

LETBY: Any volume that she's taken out of the bag -- or it would involve using a dextrose vial, which we would need to check, there's a calculation that's involved in making up 12.5%, so we would both be going through the protocol together and working out the calculations together.

JOHNSON: Yes. How long does that take?

LETBY: For 12.5% it can take quite a while. It's an unusual thing to do and there's quite a lot of processes to it.

JOHNSON: How long does it take?

LETBY: I couldn't tell you, I couldn't give you a definitive time.

JOHNSON: How many times have you done it?

LETBY: Um, not that many.

JOHNSON: Did you not know how to do it off the top of your head?

LETBY: No, I don't think any nurse would. We had to use the protocol that was on the unit.

JOHNSON: Where was that?

LETBY: Nursery 1.

JOHNSON: So right where you were?

LETBY: Yes.

JOHNSON: How long would that take to find?

LETBY: I don't think I can quantify that. Mary and I would have both needed to go through the policy against the prescription charts and then got all the equipment ready -- I can't tell you how long it would take to read the policy and work out the calculations.

JOHNSON: Can't or won't?

LETBY: No, I think it's unrealistic to think that I would be able to remember exactly how many minutes something took to do.

JOHNSON: All right. Around this time, though, you were also involved in giving medication to [Baby M], weren't you?

LETBY: Yes.

JOHNSON: How long would it have taken Mary to scrub up to be the sterile nurse?

LETBY: Um, a few minutes.

JOHNSON: If we go to tiles 135 through to 138, we see the prescription and updates that were being made to the computer relating to the giving of benzylpenicillin and sodium chloride to [Baby L], don't we?

LETBY: Yes. This is [Baby M], isn't it, sorry?

JOHNSON: I beg your pardon, it is [Baby M], you're quite right.

Those are all computerised times?

LETBY: Yes.

JOHNSON: So they're recorded by the machine when you put in the code?

LETBY: When we've both signed, yes.

JOHNSON: Yes. Plainly, Mary wouldn't have been sterile at that point if she's tapping in her code to the computer?

LETBY: No.

JOHNSON: So can we take it that it must have been after that, after 15.45, that the process of making up the 12.5% dextrose started?

LETBY: Yes.

JOHNSON: And it was while Mary was getting sterile that you sabotaged [Baby M], wasn't it?

LETBY: No.

JOHNSON: Do you remember what happened just before [Baby M] crashed?

LETBY: I recall that Mary and I were at the workbench.

JOHNSON: Do you remember what [Mother of Babies L & M] said happened in her agreed witness statement?

LETBY: Not from my memory now, no.

JOHNSON: "About 10 minutes after we left the boys, a nurse called Yvonne came running up. She said I had to go back and took me down in a wheelchair. When we got there, one of the doctors was pressing [Baby M]'s chest."

Do you remember that bit of evidence?

LETBY: Yes.

JOHNSON: And so whatever happened to [Baby M] happened just after his mum and the family had left him. Do you remember them being there?

LETBY: I can't remember specifically. I know they had visited during the day and other parents for other babies were visiting. I don't recall the exact timings, no.

JOHNSON: So another case where the parents are there but leave and then the baby collapses?

LETBY: Yes, if that's accurate, yes.

JOHNSON: And that was your opportunity to sabotage [Baby M], wasn't it?

LETBY: No. I was with Mary.

JOHNSON: And this was another case, wasn't it, where air was going in and carbon dioxide was coming out of [Baby M], but his saturations were dropping?

LETBY: Yes.

JOHNSON: Do you remember that? And do you remember that during the resuscitation notes were being made --

LETBY: Yes.

JOHNSON: -- by several of you, several nurses?

LETBY: I can't recall that specifically. I know that we were writing the drugs down, yes.

JOHNSON: You remember the paper towel, don't you?

LETBY: Yes.

JOHNSON: Because it ended up under your bed.

LETBY: Yes.

JOHNSON: And do you remember the evidence of your friends, [Nurse B] and Mary Griffith, who said their writing is on the note?

LETBY: Yes.

JOHNSON: That's why I say several of you writing down.

LETBY: Oh yes.

JOHNSON: And [Baby M] suddenly recovered, didn't he --

LETBY: Yes.

JOHNSON: -- remarkably? As quickly as he declined, he recovered.

And whilst he was desperately unwell, he had a flitting pink rash on his body, didn't he?

LETBY: I did not see any rash, no. It was not discussed at the time either.

JOHNSON: If we go to tile 171, we come to the blood gas. You remember this one, don't you?

LETBY: Yes.

JOHNSON: If we click on it we'll see [Nurse B]'s writing at 16.22 and we know that that information or that data replicates the printout that was found under your bed.

LETBY: Yes.

JOHNSON: Have you thought any more about how that printout got from what [Nurse B] says was the waste paper basket or the sensitive waste to a bag under your bed?

LETBY: No, but I know I've never taken anything out of the confidential waste bin.

JOHNSON: Well, putting the confidential waste to one side for a minute, how many blood gas records have you ever taken home?

LETBY: I can't put a specific number on it. I don't know.

JOHNSON: Is it a few?

LETBY: I don't know.

JOHNSON: Why would you take a blood gas record home?

LETBY: It's come home with the handover sheet.

JOHNSON: That's not an answer to the question.

LETBY: It is. It's got put in my pocket, like a handover sheet does, and it's come home with me.

JOHNSON: Did [Nurse B] put it in your pocket?

LETBY: I don't know that. I don't know.

JOHNSON: Are you seriously suggesting that somebody else might have put it in your pocket?

LETBY: No, I'm saying that I definitely did not take anything out of the confidential waste bin.

JOHNSON: Well, who put it in your pocket?

LETBY: Probably myself.

JOHNSON: Yes.

LETBY: It did not come from the confidential waste bin.

JOHNSON: Who took it out of your pocket?

LETBY: Myself.

JOHNSON: Who put it in the bag under your bed?

LETBY: Me.

JOHNSON: Why?

LETBY: Because I collect paper.

JOHNSON: But not bank statements?

LETBY: No.

JOHNSON: No. What's the difference from your point of view -- what makes a blood gas record collectible but a bank statement shreddable?

LETBY: That's not in context. The handover and things came back with me and they were just put to one side and not thought of again. Dealing with a household bill is in the forefront of my mind. Handover sheets aren't.

JOHNSON: What's a household bill got to do with a bank statement?

LETBY: Sorry, I assume that's the same thing: a bank statement, a bill, it's -- it's things that are happening there and then.

JOHNSON: What do you mean by "happening there and then"?

LETBY: Well, a bank statement comes through, I pay a bill, I would shred the document.

JOHNSON: Straightaway?

LETBY: I can't tell you exactly when.

JOHNSON: Are you making this up as you're going along?

LETBY: No.

JOHNSON: What was the purpose of the towel that had the resus details written?

LETBY: What was the purpose of it?

JOHNSON: Yes.

LETBY: To keep an accurate record of what was happening during the resuscitation.

JOHNSON: Yes, and for whose benefit primarily was the accurate record?

LETBY: For the medical and nursing staff that would then need to document about the events.

JOHNSON: Yes. So as it happened in the context of this particular resuscitation, it was Dr Ukoh who needed the material, wasn't it?

LETBY: It would have been all of the doctors. So the drugs would have all been prescribed retrospectively, so it could have been any of the doctors, yes.

JOHNSON: Yes, but you know, don't you, that it was Dr Ukoh to whom the responsibility fell to make the meticulous record of this resuscitation?

LETBY: Okay.

JOHNSON: Well, you do know that, don't you?

LETBY: Yes, and Dr Jayaram also wrote some notes.

JOHNSON: He did, but it's Dr Ukoh who made the principal note and I'm going to ask Mr Murphy, please, to put up tile 149, which is one of the tiles that bears this data.

Can we just scroll down, please? So "cardiorespiratory arrest" underlined double, "Pale +++". Then as we scroll down the page, please, there we see what I have just described as the meticulous record.

LETBY: Yes.

JOHNSON: "Intervention" underlined. Then in Dr Ukoh's distinctive handwriting:

"Cardiac massage commenced [by him] 16.02. Adrenaline number 1, 16.05. Number 2, 16.10. Intubation 16.10. Adrenaline number 3, 16.12. Number 4, 16.19."

And so on.

LETBY: Yes.

JOHNSON: Number 5 at 16.23 or 27. And the final, the sixth and final and apparently successful dose at 16.30; yes?

LETBY: Yes.

JOHNSON: We can show you the paper towel if you'd like to see it, but that is the data that's recorded on the paper towel, isn't it?

LETBY: Yes.

JOHNSON: Therefore it follows, doesn't it, that Dr Ukoh must have had the paper towel in his hand?

LETBY: At some point, yes, to write the notes.

JOHNSON: Yes, and if we scroll back up the note, please, we see that Dr Ukoh had the paper towel at 20.25 when he made the note?

LETBY: Yes.

JOHNSON: So at the end of the shift, wherever this paper towel from the resuscitation had been, we can say, can't we, it was in the hand of Dr Ukoh at 20.25?

LETBY: Yes.

JOHNSON: And you hung around, didn't you, to get your hands on it before you left?

LETBY: No. I had to -- I stayed late that night to cover all of the work that still needed doing.

JOHNSON: What work was that?

LETBY: I had to hand over [Baby M] and I had the two babies that I'd looked after in the daytime that I also had to hand over. I had to write the medical notes -- the nursing notes, sorry, for [Baby M].

JOHNSON: [Baby M] was handed over at the normal time, wasn't he?

LETBY: Yes, but my documentation was not completed at that point.

JOHNSON: No, you're right about that. If we go to tile 251, remembering that Dr Ukoh is making his detailed note at 20.25, we see what you were doing about 14 minutes before that, don't we, at tile 251? Is that 246 or 251, sorry? 251, yes.

So 20.11.

LETBY: That's Mary Griffith's note.

JOHNSON: Yes, okay. And where is your note?

LETBY: For [Baby M]? After this.

JOHNSON: Yes. 262. Let's go to that. 21.14. My mistake. This is your note, isn't it?

LETBY: Yes.

JOHNSON: So you'd waited an hour and a quarter after the end of your shift to make this note?

LETBY: No, I disagree. I was busy with other babies on the unit as well. I was not just sitting waiting to do this documentation.

JOHNSON: And you were hanging around to get your hand on the paper towel used by Dr Ukoh --

LETBY: No.

JOHNSON: -- and to go rooting in the bin for the printout put there by [Nurse B]?

LETBY: No. I've never rooted in the bin for anything.

JOHNSON: Because you sabotaged [Baby M] by injecting him with air, didn't you?

LETBY: No, I did not.

JOHNSON: I'm going to move on to [Baby N] now and, before the break, hopefully, deal with count 17. As before, we will start with your defence statement, please. This section of it starts at paragraph 154. I'm breaking this down to differentiate between what happened on 3 June and what happened on 15 June; all right?

LETBY: Okay.

JOHNSON: So we'll deal with the 3rd first and then we'll move on to deal with counts 18 and 19, which are on the 15th.

LETBY: Okay.

JOHNSON: At 154 you say:

"I did nothing to hurt [Baby N]. I had never encountered a baby with haemophilia before. So far as I am aware no one on the nursing staff at the Countess of Chester NNU had. It seemed that the doctors were in the same position. It appeared that no one knew specifically how to care for a baby with haemophilia. At various stages during his care the problems seemed worse because no one knew how to deal with this."

At 156:

"I can't recall who I was looking after on the night shift of the 2nd to the 3rd. I do not think [Baby N] 'collapsed' during this shift. He did require a little oxygen but otherwise there was no significant concern. I do not think it is accurate to say he was screaming for 30 minutes."

Is that what you said?

LETBY: Yes.

JOHNSON: I want to start then, please, with what you said in answer to questions from your own counsel. On 16 May you were

asked about the incident that happened at about 1.10 on the morning of 3 June and you said that you simply didn't have any memory of it; was that true?

LETBY: Yes.

JOHNSON: Have I understood your case, as it's set out in your defence statement, correctly that in reality this was a bit of a non-event, nothing serious happened at all?

LETBY: Yes, I don't believe it was a collapse that needed resuscitation, no.

JOHNSON: If we go to the first of the [Baby N] sequences of events, please, at tile 52, Mr Murphy. We get to the population distribution, which you have helpfully turned up for yourself, and the neonatal review at paragraph 2 of 6.

There we see that you had two children in nursery 4.

LETBY: Yes.

JOHNSON: One was JBR and the other AF.

LETBY: Yes.

JOHNSON: Do you remember them?

LETBY: No.

JOHNSON: You also had responsibility for AC, who's in the bottom right-hand corner of that screen.

LETBY: Yes.

JOHNSON: That child was in with parents?

LETBY: Yes.

JOHNSON: [Baby N] was in nursery 1 with Christopher Booth, who had another child in there as well.

LETBY: Yes.

JOHNSON: In nursery 2, there was a single child with Sophie Ellis. And in nursery 3, two others also with Sophie.

LETBY: Yes.

JOHNSON: And two in transitional care, being cared for or supervised by Valerie Thomas?

LETBY: Yes.

JOHNSON: Who's a nursery nurse?

LETBY: Yes.

JOHNSON: Are you suggesting that the staffing levels on this shift caused or contributed to the events concerning [Baby N] at 1.10 on 3 June?

LETBY: No.

JOHNSON: Are you suggesting that anyone's mistake or incompetence made a contribution?

LETBY: No.

JOHNSON: Do you agree that [Baby N] collapsed just after Chris Booth went on his break?

LETBY: Yes.

JOHNSON: Do you agree, as we see there, that you had two what have been described as "feeders and growers" to care for in nursery 4?

LETBY: Yes.

JOHNSON: Did you find that boring?

LETBY: No.

JOHNSON: Did you find that you had time on your hands?

LETBY: No.

JOHNSON: Do you agree that [Baby N] was in good shape at the beginning of this shift?

LETBY: Um, I think so, yes.

JOHNSON: Well, just to satisfy you that I'm not misleading you, if we go to tile 65, please. These are Nurse Booth's notes written retrospectively towards the end of the shift. As you can read for yourself there, they present an encouraging picture, don't they?

LETBY: Yes.

JOHNSON: And he is an experienced nurse?

LETBY: Yes.

JOHNSON: I want to look at what you were doing at the beginning of this shift, please. So if you turn over in the neonatal review that you have in front of you, there at lines 2 to 7 we see, first of all, the blue-coloured font, which denotes your involvement?

LETBY: Yes.

JOHNSON: And secondly, that the two children in respect of whom you were involved were the two for whom you were the designated nurse in nursery 4?

LETBY: Yes.

JOHNSON: Do we see recorded there that the first thing you did was to take observations and to feed AF?

LETBY: Yes.

JOHNSON: And you went on, having fed AF, to deal with medications, together with Christopher Booth, for the other child, JBR?

LETBY: Yes.

JOHNSON: And that was the order that you would have done things?

LETBY: Yes.

JOHNSON: Yes. I'd like to put up the feeding chart for AF, please, which is J30921. At the bottom do we see your writing, "Sleepy ++"?

LETBY: No, that's not my writing.

JOHNSON: It's not your writing? All right. If we remove that highlight, please. What do we see so far as the feed at 20.30 is concerned?

LETBY: Can you just enlarge it, please? I can't see that.

JOHNSON: Of course.

LETBY: At 20.30 the baby has had a nasogastric feed because they're asleep.

JOHNSON: Yes. 50ml?

LETBY: Yes.

JOHNSON: Prior to that -- trace aspirate, is it?

LETBY: Yes.

JOHNSON: And no entries so far as urine or --

LETBY: No, we don't do cares if the baby's asleep.

JOHNSON: Okay. 50ml takes how long, please?

LETBY: Again, I can't put a definitive number, but 10 minutes or so, 10 or 15 minutes depending on the baby.

JOHNSON: It was almost half an hour for less before [Baby K]'s collapse at 3.50.

LETBY: I think the baby's being offered a bottle feed there, were they not?

JOHNSON: Ten or 15 minutes, you say, for this one?

LETBY: Well, potentially. I can't put a -- I don't think any member of staff could put a definitive number on how long anything takes. I would say around that region.

JOHNSON: Other estimates we've been given for this sort of quantity is about 20 minutes.

LETBY: Yes, I really can't say.

JOHNSON: And as you have told us several times, when I've encouraged you, you pour 10ml at a time into a syringe; that's right, isn't it?

LETBY: Yes.

JOHNSON: Yes, a two-handed job?

LETBY: Yes.

JOHNSON: How do you do that whilst you're texting your mates?

LETBY: It's not done at the same time.

JOHNSON: No? You were texting your friends, though, right through this time, weren't you, that you were supposedly feeding this child?

LETBY: I don't know.

JOHNSON: You don't know?

LETBY: No.

JOHNSON: Have you not noticed? Well, let's look. I'm going to run through these quite quickly. If we start at tile 54, please, in the sequence of events. Remembering that you have put this down at 20.30 and it's before the medications that are given to the other two children in the nursery at 20.37, making allowances for you being 5 or 10 minutes out, you started a text conversation with your friend [Nurse E] pretty much as soon as you came on to the ward, didn't you --

LETBY: Yes.

JOHNSON: -- asking whether she was home? She responded within a minute, the next text. You responded to her again within about a minute at 20.35 (sic). And she then sent the message:

"Aren't you in handover?"

And responded to a couple more. And then at tile 61, you say:

"Caroline had gone home so I had a paper handover."

LETBY: Yes.

JOHNSON: So we're now half an hour before you feed AF; do you agree?

LETBY: Yes.

JOHNSON: You send another message to [Nurse E] 11 seconds later, tile 62. She responded within seconds, tiles 63 and 64.

Tile 66, somebody doing [Baby N]'s observations.

If we move on to tile 68, as that is going on you are telling [Nurse E], tile 68:

"We've got a baby with haemophilia"; yes?

LETBY: Yes.

JOHNSON: And you were sending -- I'll summarise these, but if you want to look at them by all means you do. In fact, we'll go through them to be fair to you. So that's tile 68 at 20.00 hours.

Tile 70, 24 seconds later:

"34."

Which is an answer to [Nurse E] asking how many weeks was [Baby N].

Tile 72, a minute later, less than a minute later:

"Everyone bit panicked by seems of things. Although baby appears fine."

She said, "Male?", you say, "Yes". She says, "Factor VIII?", you say:

"Not sure. I only know what's on the handover sheet as doctors are in with him doing a head scan."

She replies within 15 seconds -- sorry, you then send another reply/message within 15 seconds:

"Oh, had a weird Facebook message from [Dr A] earlier."

Yes?

LETBY: Yes.

JOHNSON: She replies straightaway:

"Lad with haemophilia when worked in community with Leighton on placement."

And you say:

"Ah, okay, I'll have to google it later, I don't know much about it."

20.04 -- so this is seven messages within 4 minutes; yes?

LETBY: Yes.

JOHNSON: She then says:

"Have to be careful with cannula."

Then:

"Give factor VIII or factor IX... Have as an infusion for the rest of your life [on 82]."

And you reply immediately, "Wow". She then sends you seven messages between 20.08 and 20.11, tiles 84 to 90.

Just scroll through those, please, Mr Murphy.

(Pause)

You then reply at tile 91:

"Complex condition. Yeah, 50/50 chance antenatally."

You'd been googling haemophilia, hadn't you?

LETBY: No.

JOHNSON: So when you say:

"I'll have to google it later, I don't know much about it."

You didn't intend to google it?

LETBY: Yes, but not at this point --

JOHNSON: Why not?

LETBY: -- there and then.

JOHNSON: Why not?

LETBY: Because I wouldn't have been on the computer on Google in the unit.

JOHNSON: Your phone has Google though, doesn't it?

LETBY: Yes.

JOHNSON: What was in your hand as this was all going on?

LETBY: My phone.

JOHNSON: That's where you got that information from, "50/50 chance antenatally", isn't it, from Google, Dr Google?

LETBY: No, the 50/50 chance is something that staff would know.

I didn't know any details about the condition other than it's complex and that if mum has got it then it's a 50/50 chance that the baby will have it.

JOHNSON: And the texts continued. 93, please:

"How's Z?"

You reply at 94, 5 minutes before you're about to feed AF:

"Great. In cot. Alternate B/bottle/NG feed."

Is that right?

LETBY: Yes.

JOHNSON: She replies, "Aww", and says, "Is J better?" at tile 96.

At tile 97, 4 minutes before you're about to feed AF:

"FFS. Mel asking me how to make up 12.5%."

Yes?

LETBY: Yes.

JOHNSON: Were you really unhappy with Mel?

LETBY: No.

JOHNSON: You see, in answer to questions from your counsel, you said you only swore in very extreme circumstances. What does FFS mean?

LETBY: "For fuck's sake."

JOHNSON: Yes. Were you unhappy with Mel?

LETBY: Yes, I was shocked that a band 6 senior nurse was asking me how to make up 12.5% dextrose.

JOHNSON: And you know, don't you?

LETBY: Yes.

JOHNSON: That's why I was asking you questions in relation to [Baby M]. You were telling the jury that you need to look at all the paperwork.

LETBY: Yes.

JOHNSON: You knew, but the boss, the supervisor on the shift, was asking you how to do it?

LETBY: Yes.

JOHNSON: Yes.

LETBY: When she could have looked for herself.

JOHNSON: She could. But you still told her?

LETBY: I don't know what I said to Mel.

JOHNSON: You passed her the folder. Let's look. At 98:

"Does she not know where the policy is?"

And your response. You read it, it's your answer.

LETBY: "No, I've passed her folder but now asking if can run via cannula. She needs to look herself!"

JOHNSON: You were not happy with Mel, were you?

LETBY: No, I wasn't happy with Mel being like this, no, a senior member of staff.

JOHNSON: Yes. Tile 100, a minute before you're about to feed AF:

"J is great. Still on monitor but hoping to stop over weekend."

And then your friend [Nurse E]:

"Is she in charge?"

Referring to Mel no doubt; is that right?

LETBY: Yes, I would assume so, yes.

JOHNSON: And then:

"Other J gone?"

Then according to your record, you start to feed AF, but within a minute, tile 103, what are you saying?

LETBY: "Yeah, in charge."

JOHNSON: And then 19 seconds later, what did you say?

LETBY: "No, J slow with feed but getting there."

JOHNSON: She replied within half a minute of the previous text, 106, while you are feeding AF. What did you say?

LETBY: "Oh ha ha, I was talking about J."

JOHNSON: She replied and you then sent a couple of emojis at 108.

This is all while you're feeding AF, isn't it? How do you do that? How do you text while you're doing the two-handed job of feeding a child?

LETBY: You can't.

JOHNSON: No. Well, you can, there is a way of doing it, isn't there, to feed a child very quickly? Isn't there?

LETBY: You think I pushed it in?

JOHNSON: I do. You tell the jury.

LETBY: No, I did not.

JOHNSON: How do you push it in? How would you push it in if that's what you wanted to do?

LETBY: Put the plunger on the end.

JOHNSON: Yes. That's what you were doing, isn't it?

LETBY: No.

JOHNSON: Your texts continued at 111, please. Your mind was not on AF, was it?

LETBY: No.

JOHNSON: Who was it on?

LETBY: [Nurse E].

JOHNSON: Oh come on. Who was it on?

LETBY: Well, in this specific message, [Dr A].

JOHNSON: Yes. And [Nurse E] knew you were sweet on [Dr A], didn't she?

LETBY: There was nothing between me and [Dr A].

JOHNSON: Did [Nurse E] think you were sweet on [Dr A]?

LETBY: What do you mean by "sweet on"?

JOHNSON: Had a crush on him.

LETBY: No.

JOHNSON: Well, let's see how she replied then. 112. This is about your weird -- strange, sorry, message from [Dr A] earlier:

"Did you? Saying what?"

And without me reminding you, can you remember what the next thing was she said?

LETBY: No.

JOHNSON: Really?

LETBY: Yes.

JOHNSON: Let's look. What does that mean, is it a reference to the Royal Marines?

LETBY: I don't know.

JOHNSON: You don't know? "Go commando." You don't understand what that means? Well, you understood it because if we go to 114 you found it highly amusing. So what did you understand the message to mean?

LETBY: I don't know. I can't say right now. I don't know.

JOHNSON: Do justice to yourself. What did you understand "go commando" to mean?

LETBY: I don't know.

JOHNSON: Did you think this was an army reference, you being from Hereford?

LETBY: I don't know.

JOHNSON: These messages continued. I'll just concentrate on the ones from you: 115, that's 20.39; 118, 20.40; 119, 20.41; 121, 20.43; 124, 20.44. 126:

"I don't flirt with him!"

That's a response to her message at 122 where she says:

"He's as flirty as you [as read]."

Are you still saying you don't know what "go commando" means?

LETBY: Yes.

MR JOHNSON: My Lord, that's probably a good point.

MR JUSTICE GOSS: Right, yes. We'll have a 15-minute break there then, members of the jury.

(11.49 am)

(A short break)

(12.04 pm)

MR JUSTICE GOSS: Mr Johnson, could voices be kept up, please?

MR JOHNSON: Certainly.

MR JUSTICE GOSS: Thank you.

MR JOHNSON: So we've just seen there that you were, I would suggest, texting non-stop in the time window that you were recording having fed AF. How did you manage that?

LETBY: I couldn't feed a baby while texting so it must have happened at a different time.

JOHNSON: What must have happened at a different time?

LETBY: The feed.

JOHNSON: The feed? Well, when?

LETBY: I can't answer that, I don't know.

JOHNSON: Or the alternative is, to use your phrase, you pushed it through?

LETBY: No.

JOHNSON: Is that a phrase in usage in the NNU, pushing it through?

LETBY: No.

JOHNSON: Where did you come up with it from?

LETBY: Because that's been suggested to me many times.

JOHNSON: Not that phrase, pushing it through.

LETBY: Well, how else would you describe it?

JOHNSON: I'm more interested in how you describe it.

LETBY: To put the plunger on the end would involve pushing the feed through.

JOHNSON: Yes. If we go to tile 161, please, going back to [Baby N], here we see Dr Loughnane's notes, timed at 22.55. Can you see them there?

LETBY: Yes.

JOHNSON: And by this stage, enteral feeds were being contemplated for [Baby N], weren't they?

LETBY: Can I see further down?

JOHNSON: Of course. Mr Murphy will just help us. I think we need to go on to the next page as well. Actually, I think we have just gone past it, it's my fault, being too eager. It says:

"Feed: 60ml/kg/day."

That's 10% dextrose.

LETBY: Yes.

JOHNSON: Absolutely right.

MR JUSTICE GOSS: It's right down at the bottom of the page.

MR JOHNSON: Yes, enteral feed, yes. It's where the cross is to the right.

LETBY: Yes.

JOHNSON: A sort of alpha sign, it says "enteral feeds"; do you see that?

LETBY: Yes.

MR JUSTICE GOSS: "Consider starting enteral feeds."

LETBY: "Gas normal."

MR JOHNSON: Yes, exactly.

Now, we know that [Baby N] collapsed at 01.00 hours, and that's at tile 171. If we go straight to 172 after that, please. So that's the event marked -- sorry, just pausing there for a second, we see that Christopher Booth recorded some data at 01.00 hours.

LETBY: Yes.

JOHNSON: Then if we go to his note, please, at tile 172, no doubt you remember his evidence, and we'll stay with this version of it, but we can go to the original if you want to. Whilst he was on his break, it says:

"Infant [Surname of Baby N] crying ++ and not settling. Became dusky, desaturating to 40s."

That's very serious, isn't it, 40s?

LETBY: Yes.

JOHNSON: "Responded to facial oxygen within 1 to 2 minutes."

In your experience how long does it take for a child to desaturate right down to the 40s?

LETBY: That's dependent on the child.

JOHNSON: Sure, but in general terms?

LETBY: Sometimes they can drop straightaway or sometimes they have a continual drop in their saturations that would take several minutes.

JOHNSON: Yes. That's a collapse, isn't it?

LETBY: No.

JOHNSON: So a desaturation to a life-threatening, as we've heard from experts, level of the 40s, you don't classify as a collapse?

LETBY: No, because the baby's needed facial oxygen, which is not a resuscitative measure, and has recovered within 1 to 2 minutes.

JOHNSON: So oxygen doesn't resuscitate a desaturating baby; is that your evidence?

LETBY: I don't think you can class oxygen as resuscitation. We used facial oxygen on a day-to-day basis for babies like this.

JOHNSON: Well, [Baby N] collapsed, according to -- well, let's look at Dr Loughnane's note if we can, please, tile 174. Do 173 just to put this into context.

We see Dr Loughnane coming into the NNU at 1.07.

Do you see that?

LETBY: Yes.

JOHNSON: We see her note at 1.10.

LETBY: Yes.

JOHNSON: Just reminding ourselves of what it was she noted:

"Desat. Unsettled. Increased work of breathing. Got upset. Looked mottled. Sats down to 40% in 100% oxygen. On my arrival 40% oxygen, screaming."

Do you remember the look of surprise on Dr Loughnane's face when she was sitting where you are when she saw that word that she had used?

LETBY: Yes.

JOHNSON: Screaming is very unusual, isn't it, in a child of this age?

LETBY: Yes.

JOHNSON: Chris Booth had gone on his break, hadn't he?

LETBY: Yes.

JOHNSON: And you took that opportunity to sabotage [Baby N], didn't you?

LETBY: No.

JOHNSON: This was your doing?

LETBY: No, it was not.

JOHNSON: Did you want to be in nursery 1?

LETBY: No.

JOHNSON: We have had other babies in this case screaming, haven't we?

LETBY: Yes, to some people, yes.

JOHNSON: [Baby I]. Three weeks later, one that we'll come to later today hopefully, [Baby O].

Let's move on to 15 June, please, counts 18 and 19.

It's the second sequence of events for [Baby N], please.

If we start with tile 7. These are events of 15 June, but I'm just going to start with the night shift of the 13th to the 14th; all right?

LETBY: Okay.

JOHNSON: The 13th was a Monday, the 14th was a Tuesday.

If we look at Dr Henton's note and just scroll down, this is the -- is this the preceding examination for the grand round or is this something else?

LETBY: No, that's right, this is the form that's completed on the Tuesday ready for the Wednesday ward round.

JOHNSON: Yes. Scroll down, please.

The intention was that -- can you see there's a question mark towards the top of the page, right at the right-hand edge of the page?

LETBY: Yes.

JOHNSON: So "Looking to go home this week" is the plan, isn't it, for [Baby N]?

LETBY: Yes.

JOHNSON: So that is a reflection, I would suggest, of how well he was doing at that time.

LETBY: Yes, I recall he was only in at that moment because he needed phototherapy.

JOHNSON: Yes, exactly. He was feeding well?

LETBY: Yes.

JOHNSON: If we go to tile 25, please. Your nursing note also recording the fact that he was due to go home subject to his SBR level?

LETBY: Yes.

JOHNSON: Otherwise, ready for home, as you've just told us?

LETBY: Yes.

JOHNSON: On the early part of this shift -- and this is a shift where you were looking after [Baby N], isn't it?

LETBY: Yes.

JOHNSON: [Mother of Baby N] was breastfeeding [Baby N], do you remember that?

LETBY: Yes, only from reading there that she put him to the breast, yes.

JOHNSON: We see it if we go to tile 23. This is the feeding chart for him.

LETBY: Yes, so he was predominantly bottle feeding, but he was also going to the breast as well.

JOHNSON: Yes. PTB and EBM it says, doesn't it?

LETBY: Yes.

JOHNSON: So "put to breast". So that's 11.50 on the 14th.

LETBY: Yes.

JOHNSON: So this is the day before the events that we're going to look at.

[Mother of Baby N], his mum, left at 14.20, which is tile 26. This is taken from a note that you made, the note you made at 14.20 saying that she'd left, is that right, or, Mummy visiting this morning", anyway?

LETBY: Yes, I put she aims to return later.

JOHNSON: We know she'd gone by 14.20?

LETBY: Yes.

JOHNSON: And at 17.15, so teatime, this is tile 29, you recorded the fact that [Baby N] took a 60ml bottle. That's a very good sign, isn't it?

LETBY: Yes.

JOHNSON: As he did subsequently at 21.00 hours, as a matter of fact?

LETBY: Yes.

JOHNSON: I'm going to suggest that just before you handed him over that evening, you did something to destabilise [Baby N].

LETBY: No, I did not.

JOHNSON: If we go to tile 78, we've got your friend Jennifer Jones-Key's nursing note, relating to events in the early part of the shift.

LETBY: Yes.

JOHNSON: It begins with:

"Baby very unsettled early part of night."

LETBY: Yes.

JOHNSON: So you're saying this is a coincidence that this happened just after you went off shift?

LETBY: Yes.

JOHNSON: [Baby N] was seen by [Dr A] at 01.45. If we go to tile 80, please, where he noted mottling. Do you see that?

LETBY: Yes.

JOHNSON: That was followed by some self-resolving desaturations, which we'll see in Jennifer Jones-Key's notes at tile 81, the very next tile. Do you see those?

LETBY: Yes.

JOHNSON: [Baby N]'s bloods were taken at 03.25 and 03.53; that's tiles 82 and 85. Do you accept, and we can go to them if you want to, we can go to the results if you want to, but do you accept that the lab found nothing untoward about [Baby N]'s bloods?

LETBY: Could I just see the sheet? I can't recall specifically the results.

JOHNSON: Of course. That's all right. You're fully within your rights to ask me to show you and I will. Tile 499, please. There's a text that [Dr A] sent you a few days later:

"Micro all neg."

That's microbiology, isn't it?

LETBY: Yes, that's the cultures, yes.

JOHNSON: Yes. Just going back to 03.53 then that morning, this is the morning of the day when [Baby N] collapsed twice.

I think the information about the blood cultures is also at tile 85, if we go back to that, please:

"No bacterial growth after 5 days' incubation."

LETBY: Yes.

JOHNSON: There was what [Dr A] called a "reassuring blood gas" at 03.53 or thereabouts; do you remember that?

LETBY: Yes.

JOHNSON: That's at tile 86. Blood gas may be the wrong way of putting it. This is one that actually went to the lab, isn't it? Is that right?

LETBY: Yes, sorry, I thought you were referring to the gas.

JOHNSON: There may well be a gas as well. It may be my loose use of terminology. This is a properly analysed blood sample, which gives us a reassuring picture; would you agree?

LETBY: As much as I can understand those results, yes.

JOHNSON: Well, that was the evidence from [Dr A].

LETBY: Okay.

JOHNSON: So there we are. Now, I'm then going to come to some of your text messages. You had had [Baby N] on the previous day shift.

LETBY: Yes.

JOHNSON: I'm suggesting that you did something to him before you went off duty that destabilised him in the early part of the shift; all right?

LETBY: Okay.

JOHNSON: I know you dispute that. But at 05.10 that morning, you were texting your friend [Nurse E], who was working on this shift; is that right? It's tile 92. It's probably impossible for you to carry this in your head, but let's go to tile 92 to remind you. Do you see that?

LETBY: Yes.

JOHNSON: And just to make the point that I'm seeking to make, that she was working, if we go to tile 93, please, the very next one. Okay?

LETBY: Yes.

JOHNSON: Then you asked the obvious question, if I may say so, tile 94:

"Why?"

And she replies at tile 100 a short time later and you respond:

"OMG."

Then I'm particularly going to direct your attention to tile 104, please, which is a few minutes later:

"Baby [Surname of Baby N]..."

Of course he hadn't been given a first name at this stage, had he?

LETBY: No.

JOHNSON: "Baby [Surname of Baby N] screened. Looks like shit."

We know as a matter of fact that the screening revealed perfectly normal results, don't we?

LETBY: Yes.

JOHNSON: You responded at 106 and 107:

"Really?"

And:

"Oh no."

I'm going to suggest to you that you saw this as an opportunity to sabotage him on the day shift.

LETBY: No.

JOHNSON: The reason you had done something to him before you went off the previous day shift was to give the impression that there was a progressing decline that you could take advantage of on the day shift of 15 June.

LETBY: No, that's not what happened.

JOHNSON: Right. At tile 112 your friend [Dr A] sent you a text or a Facebook message apologising for having filled the neonatal unit and talking about an issue he had with hypoglycaemia, a personal issue; is that right?

LETBY: Yes.

JOHNSON: And talking about an issue that you had with thyroid function?

LETBY: Yes.

JOHNSON: And signing off saying:

"Have a good breakfast. I think your day may be busy."

LETBY: Yes.

JOHNSON: Were you looking forward to being in nursery 1?

LETBY: I didn't know where I was going to be placed on this shift.

JOHNSON: Were you thinking that you might be in nursery 1?

LETBY: I had no idea.

JOHNSON: Let's look at tile 118. Is that where you wanted to be?

LETBY: No. But the chances are I probably would have been in nursery 1 if the unit was that busy.

JOHNSON: Well, from that point, this is 06.04, you were texting both [Dr A] and your friend [Nurse E], both of whom were working, weren't they?

LETBY: Yes.

JOHNSON: Those texts continued until 06.27 at tile 128. This was a response to [Nurse E] sending you one saying:

"Yeah, I've been about half 12 both nights."

You were talking about when you took your breaks?

LETBY: Yes.

JOHNSON: Do you remember that conversation?

LETBY: Yes.

JOHNSON: So just to put this into context, you and she were talking about when to take a break, she said that she was habitually doing it at about half 12 on nights, and you had actually said:

"Yes, I've started going 12 sometimes on nights."

LETBY: Yes.

JOHNSON: All right. I think the gist of the conversation was if you got a break in early, you were less likely to lose your break?

LETBY: Yes.

JOHNSON: All right. So that's the final text in the sequence in the document. At 06.27, you were working literally 2 minutes away, weren't you?

LETBY: Living?

JOHNSON: Living, I beg your pardon.

LETBY: Yes.

JOHNSON: Living literally 2 minutes away in Ash House?

LETBY: Yes.

JOHNSON: And this would be about the time that you were getting ready for work, wouldn't it?

LETBY: Yes.

JOHNSON: [Baby N] was in good shape, I am going to suggest.

If we look at tile 134.

LETBY: Sorry, can I just check something?

JOHNSON: Yes, please do.

LETBY: I don't think I was in Ash House at this point.

JOHNSON: 15 June, you're quite right.

LETBY: I was in my own home. I misread the year, sorry.

JOHNSON: As I did, sorry, yes. It's my responsibility to get it right. You'd moved on the 5th or 6 April.

LETBY: Yes, so I was no longer on site.

JOHNSON: Sorry, my mistake. This shows that at 7 am, [Baby N] was getting some fluids; is that right or not?

LETBY: Yes.

JOHNSON: His bowels had opened?

LETBY: Yes.

JOHNSON: Scroll down to the bottom, please. Nothing else there untoward, is there?

LETBY: No.

JOHNSON: I think we've got in jury bundle number 2, behind divider 18, so it's the big -- that's the one. It's the very first page. J19314. Do we see there that monitoring was ongoing for [Baby N]? It had been fairly intermittent on the previous -- this is a rather confusing chart, isn't it, actually, because it begins on 11 June?

LETBY: Yes.

JOHNSON: It then runs into the 12th?

LETBY: Yes.

JOHNSON: Then the 13th. Then the 14th begins at 03.45?

LETBY: Yes.

JOHNSON: Then on the day shift of the 14th we can see a block of three of your initials right at the bottom?

LETBY: Yes.

JOHNSON: Which are 11.30, 12.30 and 17.00 hours. There's then a gap until 21.00 where we see Jennifer Jones-Key's initials at 9 pm on the 14th.

LETBY: Yes.

JOHNSON: So the 15th starts pretty much where it says --

LETBY: Yes.

JOHNSON: -- 15/6/16? The first obs are at 01.00 hours, which was when [Dr A] started to get involved. Then we see 5 -- it's difficult to tell precisely what the next time is. It looks like two lots of 9 o'clock.

LETBY: I don't know. Is that 5 am and...?

JOHNSON: There certainly is a 05.00.

LETBY: It's 7. He's put two-hourly, so I'd assume it's 5, 7 and 9.

JOHNSON: So it's a continental 7 with a cross through the 7; is that right?

LETBY: It's not my writing, but I believe in the column below it says "frequency 2", which means it would be two-hourly, which would fit with 7.

JOHNSON: Okay. Thank you for that. So the first of your observations is at 09.00?

LETBY: Yes.

JOHNSON: And so far as the readings are concerned at the beginning of your shift -- of course these things have to be looked at as a trend, as we know, but so far as the 7 o'clock and 9 o'clock readings are concerned, there's nothing concerning there, is there?

LETBY: No.

JOHNSON: No. And so I'm going to suggest that when you took over [Baby N] that morning, so far as the charts were concerned at least, there was nothing suggestive of there being a problem, albeit we know there was an issue when you went in shortly after 7 o'clock.

LETBY: Yes, so I agree, at this point his observations were normal, yes.

JOHNSON: All right. Let's look at what happened as you were arriving.

Going back to the sequence of events, please, at tile 135, [Nurse E], who was working, was telling you that Minna Lappalainen was in charge for your shift; is that right?

LETBY: Yes.

JOHNSON: And if we go to the very next tile, 136, do we see that you go into the neonatal unit --

LETBY: Yes.

JOHNSON: -- at 07.10?

The next tile, 07.12, has you going from the labour ward to the neonatal unit. So that's 2 minutes later?

LETBY: Yes.

JOHNSON: What route would you have been taking to have gone through those doors at those times, do you know?

LETBY: So I've come on to the neonatal unit and then gone through to the labour ward and then have come back in.

JOHNSON: Come back in?

LETBY: Yes?

JOHNSON: Any reason why you'd go to the labour ward?

LETBY: The labour ward is where the scrubs are kept, where the toilets are. I couldn't say definitively now why I went to the labour ward but it's common practice we do go to the labour ward for things, yes.

JOHNSON: So this shows, doesn't it, that you are certainly in the neonatal unit by 07.12?

LETBY: Yes.

JOHNSON: And within 3 minutes, [Baby N], as he had done in the early hours of 3 June, had both desaturated and was crying?

LETBY: I don't recall if he was crying.

JOHNSON: Well, I'm going to suggest to you that was the evidence.

But what do you remember?

LETBY: I don't really remember. I just remember being in the doorway talking to Jennifer and I just arrived and his alarm went off.

JOHNSON: Just as you arrived?

LETBY: Yes.

JOHNSON: While she was feeding another child; do you remember that bit of evidence?

LETBY: Yes.

JOHNSON: Presumably, if she's feeding a child, her attention would be directed towards that other child in the room?

LETBY: Yes.

JOHNSON: Yes. Are you suggesting, we don't have a population chart for this one in the sequence of events, but are you suggesting that the level of provision for -- sorry, there is one in the -- if we go to the neonatal review, the NNU review. So the big black file, please.

This is actually at the start of the night shift, this one, so it's not terribly helpful. But what it shows us is that [Baby N] was in -- so it's page 2 of 13 -- nursery 3 at the start of the night shift.

If we go all the way through, please, to page 13 of 13, the very final page of this neonatal review, we see that by the end of the day shift on which the events we're about to come to happened, he was in nursery 1.

LETBY: Yes.

JOHNSON: Have you got that?

LETBY: Yes.

JOHNSON: Does that accord with your memory?

LETBY: Yes.

JOHNSON: I think the reason that the usual population chart isn't in the sequence of events is because of the difficulty of defining what happened precisely when, so what the precise position was at the beginning of the shift, but we all heard the evidence.

Just looking at the position at the end of the shift, at least, so that page 13 of 13, are you suggesting that staffing levels caused or contributed to [Baby N]'s collapse?

LETBY: Um... It was very busy, yes. I don't know what happened to [Baby N], but I know there was a lot of intervention needed by a lot of doctors and nurses.

JOHNSON: Well, are you suggesting there was a specific reason why he collapsed which would have been mitigated by there being more staff there?

LETBY: No.

JOHNSON: No. Dealing with the first collapse at about 7.15, 3 minutes after you arrived, are you suggesting that a medical or nursing mistake caused or contributed to that?

LETBY: No.

JOHNSON: So far as his subsequent collapses during the shift were concerned, are you suggesting that any mistakes or negligence or incompetence caused or contributed to those?

LETBY: I don't know whether the issues with gaining endotracheal tube access made a difference. I don't know.

JOHNSON: Why did you head straight for nursery 3 when you got in?

LETBY: To talk to Jennifer, who was my friend.

JOHNSON: But you had been, as we have just shown from references to the text messages, you'd been talking to [Nurse E], hadn't you?

LETBY: Yes.

JOHNSON: Wouldn't you normally have gone to speak to her, given what you'd been talking about?

LETBY: No, I'd spoken to [Nurse E] via text. I've gone through to speak to Jennifer.

JOHNSON: Of course, [Nurse E] had told you that, to use her phrase, "Baby [Surname of Baby N] looked like shit"?

LETBY: Yes.

JOHNSON: And you saw that as an opportunity, didn't you, to sabotage him?

LETBY: No.

JOHNSON: And you also saw it as an opportunity to make it look like you had inherited a problem from the night shift?

You sabotaged him, in effect on the night shift, by going in early?

LETBY: No.

JOHNSON: You'd set him up to fail at the end of the previous day shift and you were in there before your shift started, making a beeline for him to make it look like he came from the night shift with a problem?

LETBY: No.

JOHNSON: Do you remember Jennifer Jones-Key's evidence about the circumstances in which [Baby N] collapsed at about 7.15?

LETBY: Not in its entirety, no.

JOHNSON: What she said was that you said to her that [Baby N] looked "a bit pale". Do you remember her saying that?

LETBY: No.

JOHNSON: That's what you said to Ashleigh Hudson with [Baby I] on 13 October, wasn't it?

LETBY: Yes.

JOHNSON: Let's just look at your defence statement if we can, please. It's paragraph 158 that I'm particularly interested in, but if you want to refer to anything else you say so.

Just take a moment to look at it in its entirety so far as [Baby N] is concerned in case I miss something out.

You tell me when you've read through the whole [Baby N] section for yourself.

(Pause)

LETBY: Okay.

JOHNSON: I'm going to concentrate on -- starting at 158:

"I was on the day shift for 15 June when I went in around 7.15. I went to chat with who was present. There is nothing unusual about this, it's what we do at the start of a shift. I went to nursery 3 to speak to Jennifer Jones because she's my friend. I did not go in specifically to check on [Baby N]. Jennifer and I were talking in the doorway..."

Well, she wasn't in the doorway, was she?

LETBY: No.

JOHNSON: No:

"... the doorway to the nursery and [Baby N] was on the right-hand side. I hadn't actually had the handover at that point and we heard his monitor go off and when I looked over he was blue."

So not pale, but blue; yes?

LETBY: Yes.

JOHNSON: "He wasn't breathing. I shouted for [Dr A].

I started to Neopuff [Baby N] and Jennifer came over to help."

First of all, is that what it says?

LETBY: Yes.

JOHNSON: Secondly, is that an accurate summary of your memory insofar as it goes?

LETBY: Yes, other than it was just myself that was stood in the doorway.

JOHNSON: Why did you say Jennifer was in the doorway?

LETBY: I -- I didn't mean it as in both of us were stood in the doorway, I think it's just -- how it's come out makes it read that way.

JOHNSON: "Jennifer and I were talking in the doorway" comes out that way?

LETBY: Yes.

JOHNSON: When you were being asked questions by your own counsel, you said this:

"I went to speak to Jennifer Jones-Key as she was a good friend of mine. I was ready to start to talk to her. I was going to talk to Jennifer. I am not aware that handover had been allocated."

Do you remember saying that? What did you mean by "I was not aware that handover had been allocated"?

LETBY: So at that point the formal handover process had not happened, so I hadn't had a full handover on the unit of who was looking after who at that point.

JOHNSON: Weren't you saying, in answer to your own counsel, that you at that point didn't know who it was you were going to be looking after?

LETBY: I didn't know until I got on to the unit, no. I can't remember at what point Kate Percival-Ward told me I would be having [Baby N].

JOHNSON: You knew as soon as you got in because if we go back to tile 138:

"I've escaped being in 1, back in 3."

LETBY: Yes, so I must have been told at that point that I was going into nursery 3.

JOHNSON: Yes, as soon as you had come through the door you knew and that's why you went to nursery 3, wasn't it?

LETBY: No.

JOHNSON: Just think about that. Are you really saying that that wasn't the reason you went to nursery 3?

LETBY: No, I went to talk to Jennifer, which is what I would do on most shifts if she was in.

JOHNSON: Going back to your evidence, you then said:

"Within minutes [Baby N]'s monitor went off. I was in the doorway."

LETBY: Yes.

JOHNSON: As soon as you arrived pretty much it went off, didn't you?

LETBY: Yes.

JOHNSON: Because you sabotaged him?

LETBY: No, I did not.

JOHNSON: "I cannot remember where Jennifer Jones-Key was. She was in the room with another baby. I had not gone into the room at this point, I was in the doorway chatting."

LETBY: Yes.

JOHNSON: "She was attending to a baby that was not [Baby N]."

LETBY: Yes.

JOHNSON: "He [presumably [Baby N]] was bluish and not breathing properly."

LETBY: Yes.

JOHNSON: "Jennifer Jones-Key finished with her baby and came over to help. [Baby N] recovered, but within minutes it happened again."

This was a serious event, wasn't it?

LETBY: Yes.

JOHNSON: It happened within a minute or two of you arriving in that room?

LETBY: Yes.

JOHNSON: Just bad luck, is it?

LETBY: Yes.

JOHNSON: Do you remember saying in evidence that the first blood seen orally was "during intubation"?

LETBY: Did I say that?

JOHNSON: Yes, in evidence, in answer to questions from your counsel.

LETBY: I can't recall now.

JOHNSON: Well, do you remember? You were asked lots of questions about the I-Gel airway.

LETBY: Yes.

JOHNSON: Do you remember that?

LETBY: Yes.

JOHNSON: And you were taken to your interviews, the detail of your interviews, and Mr Myers was seeking to elucidate from you what you meant, what you were saying to the police in interview about when it was you had seen blood in [Baby N]'s mouth.

LETBY: Yes.

JOHNSON: Do you remember that?

LETBY: Yes.

JOHNSON: And you understand the importance of this point, don't you?

LETBY: Yes.

JOHNSON: Yes. Because if there was blood before anybody tried to intubate [Baby N], that potentially could be worse for you, couldn't it?

LETBY: Yes.

JOHNSON: Because it might suggest, and you know that I will suggest, that you did something to cause the bleeding.

You understand that's the prosecution case, don't you?

LETBY: Yes.

JOHNSON: So the swelling to the epiglottis, the swelling to the throat, the blood coming up from under the vocal cords, all features that we have seen in other cases, but are features of [Baby N]'s case on this day?

LETBY: Yes.

JOHNSON: And you know, don't you, that we suggest that was because you sabotaged him?

LETBY: Yes.

JOHNSON: Right, okay. So we know where the battle lines are, if you like. You understand, don't you?

LETBY: Yes.

JOHNSON: Okay. What I want to understand before the break, if we can, please, is I'd like you to tell us when you are saying blood was first seen orally on 15 June in the case of [Baby N].

LETBY: So the only time I remember definitively seeing it was about 3 o'clock that afternoon with the I-Gel airway.

I can't remember specifically when else I've seen blood.

JOHNSON: But of course you've, in preparation for me asking you questions, looked at the notes, haven't you?

LETBY: Yes.

JOHNSON: You've looked at what you have said in interview?

LETBY: Yes. In interview I was referring to the 3 o'clock incident.

JOHNSON: Well, I know that's what you say and you will understand that we don't accept that. You understand that, don't you? Yes. So when you say "definitively", what do you mean by that in this context?

LETBY: Sitting here now I cannot say with any 100% accuracy when I saw something.

JOHNSON: Okay. What about if you've written it down at the time, what does that tell us definitively?

LETBY: That that's what happened around that time.

JOHNSON: Right. So if you have written down in the paperwork that you saw blood much earlier than the I-Gel airway was used, would you accept that as definitive evidence from you that there was blood at that time?

LETBY: Yes, although it may not have been seen by myself, that might have been information I was told that I've then written on to the chart.

JOHNSON: And who else would be giving -- before we start going through these records, which we will, who else was in a position to be telling you, the designated nurse, about blood?

LETBY: The medical team that were dealing with [Baby N].

JOHNSON: Right. So initially, at least on this particular shift, are we talking about [Dr A]?

LETBY: Yes.

JOHNSON: And do you accept what [Dr A] says about it or do you not accept it?

LETBY: I can't recall all of his evidence now, but if that's what he says, I can't -- I can't say whether that's true or not. I didn't look down his throat.

JOHNSON: Well, [Dr A]'s not one of the Gang of Four, is he?

LETBY: No.

JOHNSON: Did [Dr A] have it in for you as far as you knew?

LETBY: No.

JOHNSON: He was a good friend of yours, wasn't he?

LETBY: Yes.

JOHNSON: Who you say you loved as a friend?

LETBY: Yes, as time went on, yes.

JOHNSON: Do you accept what [Dr A] says about this initial desaturation about the handover time, about it being concerning, to use his word?

LETBY: Yes.

JOHNSON: Can you remember what you said to the police about what you remembered about these events when they questioned you about them?

LETBY: No.

JOHNSON: Let me just remind you. If we go to the interview bundle number 2, please. It's the [Initials of Baby N] section and it's the first interview, page 7, please.

LETBY: Which page is it, sorry?

JOHNSON: [Initials of Baby N] 7, please. Just to put this into context, what the police are doing here is showing you the chart that we just looked at, which is J19314; okay?

LETBY: Okay.

JOHNSON: Which I think is open just below -- you're in the wrong bundle. Sorry, you've been given the wrong bundle.

It's bundle number 2. Sorry, it's the interviews.

LETBY: I am in 2.

JOHNSON: I beg your pardon. My eyesight. Old age. Are you on page [Initials of Baby N] 7?

LETBY: Yes.

JOHNSON: Good. Sorry, my fault.

Can you see right at the top of the page it says:

"Do you recognise the signatures before you've signed your initials"?

LETBY: Yes.

JOHNSON: You say:

"Yeah, I think that's Jenny Jones."

What you're referring to at that stage, this is part of the evidence, I think, when these interviews were read out, which someone may or may not have written down, but this is a reference to page J19314. Okay?

LETBY: Okay.

JOHNSON: That's the chart we just looked at, which I said was a bit confusing.

LETBY: Yes.

JOHNSON: It's on the screen now. You say:

"Oh, did I have the baby from half 7?"

About halfway down. Do you see that?

LETBY: Yes.

JOHNSON: They say:

"Something's happened for him to be moved."

And you say:

"Something's changed there because he's gone from just temperature obs to full obs."

Which is what we looked at before at 01.00 hours.

LETBY: Yes.

JOHNSON: And then over the page, you say:

"Answer: The first observations are at 9 o'clock.

"Question: Do you remember when the first attempts to intubate were?"

You say no. And you say:

"When I've taken over at 9 o'clock, the line was occluding."

LETBY: Yes.

JOHNSON: And you then go on to refer to further entries on the chart.

LETBY: Yes.

JOHNSON: Had you completely forgotten about the serious or concerning, to use [Dr A]'s word, concerning desaturation at 7.15?

LETBY: Yes, at this point I did not connect [Baby N] with this baby. No, I was reading purely from the notes and it looked like at 9 o'clock I had taken over care.

JOHNSON: I see.

LETBY: I had no memory of who [Baby N] was at this point.

JOHNSON: Let's just look at your nursing note, please, at tile 151. As always, if you want to go to the original behind this, which has more information in it, you please say so.

LETBY: Okay.

JOHNSON: But let's just look at what we've got on this screen for now. This is you writing it up at 13.53 that afternoon --

LETBY: Yes.

JOHNSON: -- when things would have been very fresh in your mind, wouldn't they?

LETBY: Yes.

JOHNSON: You would not have forgotten about the desaturation at 7.15 that afternoon, would you?

LETBY: No.

JOHNSON: No. What you write is this:

"Written for care given from 07.30 to present."

So you're not referring at all to events at 07.15, are you?

LETBY: No, because at that time he was still under the care of Jennifer Jones.

JOHNSON: Well, he was, but who discovered them?

LETBY: Well, we were both there.

JOHNSON: Who discovered him?

LETBY: We both heard his monitor, I went over to him.

JOHNSON: Mm. He was looking a bit pale, your words to her. This has echoes, doesn't it, of [Baby I] and Ashleigh Hudson?

LETBY: I don't recall commenting that he was pale, but...

JOHNSON: Well, just thinking about [Baby I] for a second, you wrote that one up, didn't you, in the nursing notes?

LETBY: Yes.

JOHNSON: Why?

LETBY: Because that was a serious life-threatening collapse that required resuscitation. I then took over [Baby I]'s care.

JOHNSON: This was concerning, that the child continued to collapse throughout the day and you then took over his care, so why didn't you write it up?

LETBY: I don't feel that it's the same context at all of having some desaturations compared to finding [Baby I] and her needing full resuscitation.

JOHNSON: The answer, I am going to suggest to you, is because you were hoping to create the impression in the paperwork that these were all events that had happened before you arrived.

LETBY: No, I disagree.

JOHNSON: It says:

"Infant transferred to nursery 1 on handover. Mottled, desaturating, requiring Neopuff and oxygen. Capillary refill 3 to 4 seconds. Cold to touch."

You had written out the events of your arrival in nursery 3, hadn't you?

LETBY: Yes.

JOHNSON: Why?

LETBY: I've taken over [Baby N]'s care from 7.30.

JOHNSON: Let's go to the original, please. What time is this note written?

LETBY: 14.10.

JOHNSON: "Decision made to intubate. Drugs given as prescribed. Unable to intubate."

What does the next few words --

LETBY: "Fresh blood noted in mouth and yielded by suction ++."

JOHNSON: Who does the suction?

LETBY: Doctors during intubation.

JOHNSON: Not the nurses?

LETBY: No.

JOHNSON: You were there.

LETBY: Yes, I was there.

JOHNSON: Do you remember me asking you when you first noted blood?

LETBY: Yes.

JOHNSON: What does this note, made at 13.53, suggest was the time you first saw blood?

LETBY: After attempts to intubate.

JOHNSON: Yes, but the I-Gel doesn't come into play until a good hour or two after this note is made, does it?

LETBY: No.

JOHNSON: No. What's your explanation for that?

LETBY: For what?

JOHNSON: The disconnect between the evidence you gave to the jury that the first time you saw blood was when the I-Gel was used and the note that you made between 13.53 and 14.10, please?

LETBY: The first time I definitively remember seeing blood was prior to the I-Gel insertion.

JOHNSON: The first time you definitively remember seeing blood was prior to the I-Gel insertion? Well, this is prior to the I-Gel insertion, isn't it?

LETBY: Yes.

JOHNSON: It's a good hour before the I-Gel insertion.

LETBY: Yes.

JOHNSON: [Baby N] was bleeding from when you first got involved with him that day, wasn't he?

LETBY: No.

MR JOHNSON: We'll come back to this after the break, please, my Lord.

MR JUSTICE GOSS: Right. 2.15 then, please, members of the jury.

(1.02 pm)

(The short adjournment)

(2.15 pm)

MR JUSTICE GOSS: Yes, Mr Johnson.

MR JOHNSON: Thank you, my Lord.

You will remember just before the break that I was asking about the nursing note that you made at just before 2 pm.

LETBY: Yes.

JOHNSON: One of the issues that we began to deal with before the break was the point at which you first saw blood --

LETBY: Yes.

JOHNSON: -- and the suggestion from the prosecution that that was before [Dr A] tried to intubate [Baby N] --

LETBY: Yes.

JOHNSON: -- and your counterposition that the first you were aware of any blood was some time immediately before the I-Gel insertion.

LETBY: From my memory, yes. I know there was blood before that.

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: Well, there was a lot of blood before that, wasn't there, before the I-Gel? Wasn't there?

LETBY: Yes.

JOHNSON: Yes. Because you will remember, and I think we dealt with this as well before the break, that I reminded you about a series of questions and answers that passed between you and your counsel where there was an analysis of the interview and what you meant by saying various things at various times with relation to the appearance of blood.

LETBY: Yes.

JOHNSON: Am I correct in suggesting that the position you reached as a result of the questioning and answers you gave was that the blood was just before the I-Gel?

LETBY: Yes, because that's what I thought was being referred to in this interview, yes.

JOHNSON: Right. And for example, one of the parts of the interview that was referred to was page 20, [Initials of Baby N] 20. So it's the first of the [Baby N] interviews on 10 June 2019, so this is your second arrest.

LETBY: Yes.

JOHNSON: So this is a case that made itself known to you or an accusation that made itself known to you on your second arrest, not your first.

LETBY: Yes.

JOHNSON: If you look at page 20 the officer is asking you about blood and says:

"So if what you just said, Lucy, you're happy in your memory that there was blood present before attempts to intubate, is that what you've just said without the tube, before you've tried to put in the tube?"

And you say yeah.

LETBY: Yes.

JOHNSON: Then this question:

"And that's from your memory because it's not clear in the notes there, is it?"

And you say:

"No, but the chart as -- because I have documented it here, haven't I, at 9 o'clock, when I've done his cares, that there's fresh blood when I have checked the tube?"

That's a reference to the NG tube, isn't it?

LETBY: Yes.

JOHNSON: But what you are saying there clearly, I suggest, is that before 9 am on 15 June, you saw blood.

LETBY: No, I disagree. The intubation I'm referring to is the 15.00.

JOHNSON: Where does it say that?

LETBY: The whole line of questioning is regarding the I-Gel.

When I get further on...

JOHNSON: Oh, I see.

LETBY: I don't refer to the handover intubation at all.

JOHNSON: The use of an I-Gel airway isn't an intubation --

LETBY: It is, yes.

JOHNSON: No, it isn't.

LETBY: It is. It's a secure airway.

JOHNSON: It doesn't go through the vocal cords into the lungs, does it?

LETBY: I'm not sure exactly where it goes, but it's a mechanical airway that we never usually use, so it is a form of ventilation.

JOHNSON: We heard evidence from Dr Brearey and other doctors it is not an intubation.

LETBY: Okay.

JOHNSON: You disagree with that?

LETBY: Yes, I was referring to it as an intubation, yes.

JOHNSON: I see. Do you remember [Dr A]'s evidence about all this, about events from about shortly after 7 am?

LETBY: Not in the entirety, no.

JOHNSON: He said to the jury that he saw blood in [Baby N]'s throat before he started the intubation process at about 8 o'clock. Do you remember that?

LETBY: Yes.

JOHNSON: Let's go to tile 155, please.

MR MYERS: My Lord, I'm trying to restrain the times I get to my feet to point out issues that we have. This is one when I cannot because the evidence of [Dr A] went beyond that when taken as a whole. And understanding where this questioning is going, it's only fair the witness is reminded of what he said in total because that isn't the only thing that he said and he was taken to his statement in cross-examination where he made it plain he couldn't say if it was before the attempt to intubate and he accepted in examination, cross-examination, it was possible it may have been there before he actually commenced the procedure or may have been there afterwards, it could be either way.

MR JUSTICE GOSS: Yes.

MR MYERS: I raise that because it wouldn't be right for the defendant to be cross-examined as if it's just the one statement as just presented. I'm just anxious that that is clear.

MR JUSTICE GOSS: Yes. Mr Johnson.

MR JOHNSON: Well, I hadn't finished questioning, but anyway. It's very helpful.

MR MYERS: I'm not sure where it's going and the witness doesn't know where that's going. It was simply left on the basis it was seen before and I did wait to see if anything was added and it wasn't.

MR JUSTICE GOSS: Right.

MR JOHNSON: Well, if Mr Myers waits a little longer, his patience may be rewarded. Tile 155, please.

This is you co-signing with Bernadette Butterworth for morphine, isn't it?

LETBY: Yes.

JOHNSON: At 156, sodium chloride?

LETBY: Yes.

JOHNSON: And at 158, atropine?

LETBY: Yes.

JOHNSON: And at 160, suxamethonium?

LETBY: Yes.

JOHNSON: And these are all drugs that are given within a six-minute period?

LETBY: Yes.

JOHNSON: For what purpose?

LETBY: Intubation.

JOHNSON: There was an attempt to intubate at about 8 am?

LETBY: Yes.

JOHNSON: Do you remember [Dr A] also saying he saw swelling at the back of [Baby N]'s throat?

LETBY: Yes.

JOHNSON: Do you remember him saying, "It must have been unusual for me to have remembered it"?

LETBY: Yes.

JOHNSON: And the clear import of his evidence, I am suggesting to you, is that he was saying this was all before he intubated or tried to intubate [Baby N]. Do you agree?

LETBY: No, I don't think I can comment on what [Dr A] saw.

JOHNSON: Well, let's look at tile 171. Your note, please:

"Unable to intubate. Fresh blood noted in mouth."

That's what we referred to before.

LETBY: Yes.

JOHNSON: What does that mean?

LETBY: The doctors were unable to intubate and fresh blood was noted in the mouth and on suction when trying to insert the tube.

JOHNSON: Yes. They can't see, can they? The doctor cannot see because of the blood.

LETBY: Well, I think you'd have to clarify that with them because that's not a view that I would see as a nurse, so I can't possibly comment on what exactly [Dr A] saw.

JOHNSON: Do you agree that Professor Kinsey, in effect, eliminated the possibility of a spontaneous haemorrhage?

LETBY: Yes.

JOHNSON: Do you remember what you said to [Father of Baby N]?

LETBY: No. At what point?

JOHNSON: At about this point.

LETBY: No.

JOHNSON: [Father of Baby N], you will remember, is the father of [Baby N].

LETBY: Yes.

JOHNSON: And he said this in a statement that was read to the jury:

"I was at work. I then received a phone call from [Baby N]'s nurse, Lucy."

LETBY: Yes.

JOHNSON: "Lucy said that [Baby N] had been a bit unwell in the night, but she said he's okay now. I told Lucy that [Mother of Baby N] [and that's [Mother of Baby N], [Baby N]'s mum] would be in in a bit to see him as usual and that was that. She [this is you] didn't give me any other information. I didn't get the impression that he was still unwell or that we needed to be concerned. About 10 minutes later, [Mother of Baby N] rang me. We needed to go to the hospital."

You rang [Father of Baby N], didn't you?

LETBY: No.

JOHNSON: Why was that evidence agreed?

LETBY: I don't know that it is agreed.

JOHNSON: It was.

LETBY: Okay. I think there's a written note by Bernadette Butterworth to say that she spoke with the parents.

JOHNSON: Is there? You think about that.

LETBY: Well, I don't recall speaking to the parents myself.

JOHNSON: Tile 169, please, as you've brought it up. Can we have the original, please?

The bottom right-hand corner, Mr Murphy, thank you very much.

So this is Bernadette Butterworth's note, is it?

LETBY: No, it's my note referring to Bernadette Butterworth and I would not have put her name in there had that not happened.

JOHNSON: Because you don't make false notes?

LETBY: No.

JOHNSON: "I think there's a note by Bernadette Butterworth to say that she spoke with the parents."

That's what you just said. Was that true?

LETBY: No, I said "I think" and I understand now that this -- it was myself that wrote that.

JOHNSON: Oh, you knew very well it was you that wrote it, didn't you?

LETBY: I do now I am seeing it, yes.

JOHNSON: Because you study these papers in advance of me asking you questions about them, don't you?

LETBY: I look at them, yes.

JOHNSON: Yes. What have you written at 14.10 to 14.12 that afternoon?

LETBY: "Parents were contacted by Staff Nurse Butterworth during intubation. Both mobile phones switched off and no answer on landline. Message left. Call returned shortly after and parents were asked to attend. Have been present since. Both understandably upset. Support given and fully updated by myself, Consultant Saladi and medical team. Aware of support needed and plan of care. Mummy expressing, milk frozen."

JOHNSON: "I was at work and then I received a phone call from [Baby N]'s nurse, Lucy. Lucy said that [Baby N] had been a bit unwell in the night but he's okay now."

That was the agreed evidence. How does that agreed evidence live with your note?

LETBY: There's a difference in recollection.

JOHNSON: No, because the agreed evidence is the truth.

LETBY: Well, I can't answer that.

JOHNSON: Well, the answer is your note is a lie.

LETBY: No, it's not.

JOHNSON: In her agreed evidence, [Mother of Baby N] said this:

"On the 12th day, 14 June, we were told by a consultant that he [that is [Baby N]] was able to go home the next day."

So that's 15 June, the day we're talking about, isn't it?

LETBY: Yes, she was talking -- on the 14th, yes.

JOHNSON: "[Baby N] was in nursery 4 by that point. I was" --

LETBY: I disagree.

JOHNSON: Sorry?

LETBY: He was not in nursery 4.

JOHNSON: Well, she's got the nursery number wrong:

"I was due to go and collect him on 15 June around 8 am. I had a phone call from [Baby N]'s dad, [Father of Baby N], to say that there had been an issue at the hospital and [Baby N] had had a bleed and was unwell."

This is information coming from you to [Father of Baby N], to [Mother of Baby N].

LETBY: No, I disagree.

JOHNSON: But it's agreed evidence.

LETBY: Well, I disagree with it now.

JOHNSON: It's yet another account from a parent that has been agreed and you are now saying is untrue; is that right?

LETBY: Yes, I have no recollection of speaking to the family on the phone and I would not have put Bernadette Butterworth's name in there had that not been the case.

JOHNSON: Were you sitting in court when this agreed evidence was read to the jury?

LETBY: Yes.

JOHNSON: What did you do about it?

LETBY: Well, with respect, there's been a lot of evidence gone over and I've relied on my legal team. I can't say any more than that.

JOHNSON: We've seen you firing out Post-it notes from the dock.

LETBY: Yes, which I think I'm entitled to do.

JOHNSON: You are, absolutely. Absolutely you are. That's the whole point of you being here. Have you raised an issue with these agreed statements being read?

LETBY: I'm not sure, no.

JOHNSON: Well, is the answer no?

LETBY: I can't say now whether it's something I've ever raised with my legal team, no.

JOHNSON: Well, if you did raise an important issue like that and they didn't pursue it with the court, what would you do?

LETBY: Well, I think we're getting beyond that. I don't want to comment on what-ifs and buts, I don't know.

JOHNSON: Well, we've heard you interrupting [Mother of Babies E & F], for example, when she gave evidence, saying you couldn't hear. We saw you interrupt [Dr A] when he first came to give

evidence and you tried to get out through the back door of the dock.

LETBY: Yes, because I felt unwell.

JOHNSON: No, no. You didn't like hearing your boyfriend giving evidence, did you?

LETBY: That's not fair.

MR MYERS: If there is an issue with this, there is another way of dealing with it. If there's something that should have been put to this witness that wasn't, and if my learned friend wishes to make the issue that he does, and it may be something we have to look at carefully, and it may not be right to seek to push this point with Ms Letby in the way that it is being put, and therefore it's something that we need to consider carefully. But with respect, it seems to me inappropriate to push this in the direction it's going, asking her about what she has or hasn't said to those who are representing her.

MR JUSTICE GOSS: Yes. Well, Mr Johnson, the point has been made.

MR JOHNSON: Yes.

MR JUSTICE GOSS: Mr Myers asks for the opportunity to consider this and therefore could you move on to the next thing?

MR JOHNSON: Yes.

MR JUSTICE GOSS: I'm not barring you completely, I'm just saying at this stage we'll move on to another topic.

MR JOHNSON: Yes. I had made my point and I was moving on, but I will continue, my Lord, thank you.

You told [Father of Baby N] two lies, didn't you?

LETBY: No.

JOHNSON: First of all, you minimised how ill [Baby N] was and said he'd just been "a bit unwell in the night", and secondly you told him that [Baby N] was "okay now". That's right, isn't it?

LETBY: No.

JOHNSON: And the third lie is you attributing the conversation to Bernadette Butterworth.

LETBY: No.

JOHNSON: And this is part of your pattern of behaviour, distancing yourself from and making misleading records?

LETBY: No. Bernadette's name is in there because that's what happened and all staff on the unit have access to those notes. If anybody disagreed with that at the time they could have looked at that.

JOHNSON: Well, that would presuppose that Bernadette Butterworth had a reason to go and look at the note, wouldn't it?

LETBY: A lot of the nurses look at any of the notes at any point. That's up to them.

JOHNSON: Yes, absolutely. But do you ever suspect your colleagues of making false notes about you?

LETBY: No.

JOHNSON: Do you spend your time reading the notes of other children?

LETBY: Yes, sometimes, yes --

JOHNSON: You do?

LETBY: To familiarise myself with who else is on the unit, yes.

JOHNSON: Is that because you're so busy you haven't got any time?

LETBY: I don't understand. That doesn't make sense.

JOHNSON: Well, you have repeatedly told us how busy things are on the unit.

LETBY: Yes.

JOHNSON: Have you got time to be reading the notes for children who you are not caring for?

LETBY: Yes, at times. It's important all staff know about the babies on the unit.

JOHNSON: If we can go to page 29 of your interview, please, [Initials of Baby N] 29.

This is the interview of 10 November 2020.

This is what you were saying in 2020 about this telephone conversation; all right?

LETBY: Yes.

JOHNSON: The officer:

"Question: I've got a statement from [Father of Baby N] who's the father of [Baby N] and he evidences a telephone call, which he received from you on 14 June, saying that [Baby N] had been unwell in the night but he was doing okay now. He says 10 minutes later, [Baby N]'s mum called him, saying [Baby N] was poorly and they needed to go to the hospital. Do you understand what he said there, Lucy? So he's saying that he received a call --

"Answer: And it was from me?

"Question: Why did you tell [Father of Baby N] that [Baby N] was doing okay?"

LETBY: No, I said, "Was it from me?"

JOHNSON: Sorry, you're quite right:

"Answer: And was it from me?

"Question: Why did you tell [Father of Baby N] that [Baby N] was doing okay?

"Answer: I don't recall the conversation."

LETBY: No, I don't.

JOHNSON: You've been on notice now for -- since then that that's what [Father of Baby N] was saying, haven't you?

LETBY: Yes.

JOHNSON: When this call was made, did you know that Jennifer Jones-Key was going to say that you had been at the scene of the first collapse?

LETBY: I didn't make the phone call.

JOHNSON: You carried on making false entries, didn't you, in the paperwork?

LETBY: No.

JOHNSON: Let's see if you're saying these are true or untrue.

Can we look at J19316, please, which is in the hard copy bundle behind divider 18. It's probably in the sequence of events at -- I imagine it's tile 173.

It's the top line that's actually written on there, please, Mr Murphy. 9 o'clock. 09.00, sorry.

That's your writing, isn't it?

LETBY: Yes.

JOHNSON: Is what you have written there true?

LETBY: Yes.

JOHNSON: What does it say?

LETBY: So I've written the drip reading at 9 o'clock and said that it was occluding. I can't see what the 10ml is in reference to.

JOHNSON: Right. What does the 10 o'clock reading say?

LETBY: Again it's got the drip readings, then it says:

"1ml fresh blood. Not passed urine. Green stool."

JOHNSON: 1ml fresh blood, which column is that in?

LETBY: I'd have to see the top of the page, please.

JOHNSON: Yes. The "NG/vomit/asp" column, is it?

LETBY: Yes, I believe it would be, yes.

JOHNSON: How did that blood find its way, coming up out of the NG, as a vomit or as an aspirate?

LETBY: I can't say from reading that which it was, whether it was a vomit or an aspirate.

JOHNSON: [Baby N] had had a serious desaturation at about 07.15 or thereabouts; is that right?

LETBY: Yes.

JOHNSON: [Dr A] had tried or made preparations for intubation at about 08.00?

LETBY: Yes.

JOHNSON: At 10.00 hours -- well, you knew that [Baby N] had haemophilia?

LETBY: Yes.

JOHNSON: And at 10.00 hours, you were making a note that there was a millilitre of fresh blood in the NG/asp/vomit column?

LETBY: Yes.

JOHNSON: What did you do about it?

LETBY: I can't say from my memory now.

JOHNSON: Well, you know what's in or isn't in the notes, don't you?

LETBY: No, I don't recall my notes word for word, no.

JOHNSON: Well, given that background and given the particular issue that this child had, what should you have done about fresh blood in the mouth?

LETBY: I don't know that it was in the mouth. Fresh blood would be escalated to somebody, whether that's the nursing staff or medical.

JOHNSON: So who did you escalate it to?

LETBY: I don't know.

JOHNSON: There is no record of you escalating it to anybody, is there?

LETBY: No, if that's the evidence, no.

JOHNSON: Well, I am suggesting to you that there is no record anywhere of you escalating this issue with this child at about this time.

LETBY: Okay.

JOHNSON: Do you dissent from that?

LETBY: I agree there's no written record, yes. Whether there is any verbal conversation, I can't say.

JOHNSON: Well, this was potentially very serious, wasn't it?

LETBY: Um, 1ml of fresh blood is not -- is not normal but it's not a life-threatening event.

JOHNSON: For a child with haemophilia who had desaturated to very low levels a couple of hours earlier, it was serious, wasn't it?

LETBY: I don't agree that 1ml of fresh blood would require crash calling or anything like that.

JOHNSON: Would it be a concern?

LETBY: Yes.

JOHNSON: And to whom would the concern be escalated?

LETBY: A senior member of the nursing team or medical staff.

JOHNSON: Let's go to -- so this is 10 o'clock. Let's go to tile 185. Dr Saladi was on a ward round at 10 o'clock, wasn't he?

LETBY: Yes.

JOHNSON: There was no mention of this to Dr Saladi, was there?

LETBY: I can't say.

JOHNSON: Is it in his notes?

LETBY: I don't recall his notes, but if you've said that it's not in there, then no.

JOHNSON: Well, let's look. Would you like to see the original?

Any mention of blood? So:

"Attempt intubation x3, blood in oropharynx..."

MR JUSTICE GOSS: "Saw some clear", I think.

MR JOHNSON: Yes. No mention of anything coming up the NG tube, is there?

LETBY: No.

JOHNSON: Dr Henton's note, tile 188, please. Is this a note made on the day, do you know? At the bottom, possibly, it will have a time.

LETBY: I think usually this is -- oh, so the grand round form would be present, yes, at the time, yes. "See written notes", yes.

JOHNSON: Yes. Again no mention of this blood up the NG tube at about this time, according to your note?

LETBY: No.

JOHNSON: That is why I am suggesting that at 10 o'clock, if there really was blood, you would have done something about it and the fact that you haven't done anything about it shows that you have invented the blood at 10 o'clock.

LETBY: No, I disagree.

JOHNSON: Well, what did you do?

LETBY: I can't tell you exactly what I did.

JOHNSON: This is all designed to give the impression of an ongoing problem in a child who had haemophilia, isn't it?

LETBY: No.

JOHNSON: Do you remember the explanation that you gave to the police about it?

LETBY: No.

JOHNSON: Sorry, did you answer?

LETBY: No, I don't remember, no.

JOHNSON: Well, let's look at page [Initials of Baby N] 10, please. Let's go back to [Initials of Baby N] 9 because it puts it into context. Do you see there, about a third of the way down, the police were asking you about this very point?

LETBY: Yes.

JOHNSON: You were able to answer the question then that I asked you earlier:

"I've obviously checked his tube and 1ml has come out."

LETBY: Yes.

JOHNSON: "Question: Would that be concerning, fresh blood?

"Answer: Yeah.

"Question: You explain what you did about it, Lucy.

"Answer: I don't remember."

Is that right?

LETBY: Yes.

JOHNSON: Then over the page, just at the bottom of that page:

"Question: Just in your experience, Lucy, the fresh blood, what can cause that?"

"Answer: Is this when he was first born?

"Question: No, this is on the 15th.

"Answer: So any sort of trauma to the airway."

Is what you said. Is that right?

LETBY: Yes.

JOHNSON: "He's having some sort of an abdominal issue, an abdominal bleed, something like that. If the tube's been -- the NG tube's been inserted forcefully, it can cause a bit of trauma going down."

LETBY: Yes.

JOHNSON: Is that true?

LETBY: Which part? Well, yes, it's all true, yes.

JOHNSON: Is it true that the NG tube can cause bleeding in the throat or the mouth?

LETBY: Yes, a small amount, yes.

JOHNSON: Do you remember it being suggested to Dr Evans on your behalf that that was a ridiculous suggestion, that the NG tube -
-

LETBY: For a very large amount of blood, yes. Yes.

JOHNSON: Do you remember the expert evidence of Elizabeth Morgan, the expert nurse, which was agreed?

LETBY: Not in the entirety, no.

JOHNSON: That any unusual occurrence should have been escalated verbally to the medical staff, particularly where it was relevant to a particular issue to which the blood was relevant. So a haemophiliac child bleeding.

LETBY: Yes.

JOHNSON: You didn't escalate it, did you?

LETBY: I cannot say that I didn't escalate it verbally.

JOHNSON: You were telling your friend [Dr A] about it, though, weren't you? Do you remember?

LETBY: Yes, I remember at some point [Dr A] asked me how he was.

JOHNSON: Let's just look at tile 199, please, which is a Facebook message you sent him at 11.29. Read that out, please.

It's your message.

LETBY: "Small amounts of blood from mouth and 1ml from NG. Looks like pulmonary bleed on X-ray. Given factor VIII. Wait and see. Apnoeas have improved and gases good. Colour and perfusion still not great. If deteriorates, will try to intubate."

JOHNSON: As a matter of fact, the X-ray had been at 09.08, which is well before your record of the blood, isn't it?

LETBY: The 10 o'clock, yes.

JOHNSON: Yes. If we go back to tile 176, please. This is the reason for the -- this is an X-ray taken at 09.08, reported on by Dr Wright, the radiologist:

"Presumed haemophilia A. Profound desaturation and blood at oropharynx on intubation [query] pulmonary haemorrhage."

That was the reason for the X-ray; is that right?

LETBY: Yes.

JOHNSON: So that was [Dr A]'s reason for commissioning the X-ray?

LETBY: Yes.

JOHNSON: But Dr Wright's report says the appearance is more in keeping with infection than with pulmonary haemorrhage.

LETBY: Yes. This information comes some time after the X-ray, though, it's not available at that time.

JOHNSON: So I'm suggesting to you that you were creating the impression with your colleagues that [Baby N] had a problem which, if it existed at all, was being caused by you.

LETBY: No. Can you rephrase that, please?

JOHNSON: Yes. I'm suggesting two things. Either there wasn't a problem at all and you were making it up, as evidenced by the fact you didn't report it through the normal chain of command, or, if there was a problem, you were causing it.

LETBY: No, I disagree with both.

JOHNSON: Do you remember [Baby N]'s parents arriving?

LETBY: Not in clarity, no.

JOHNSON: Well, you referred to it in a text message you sent to your friend [Nurse E].

LETBY: Yes.

JOHNSON: Tile 232. Sorry, that's her question to you, actually, 232. 13.51, please:

"Baby G's parents been in?"

Asks your friend.

LETBY: Yes.

JOHNSON: And your response comes 3 minutes later at 13.54, tile 235, please. So [Nurse E], had she gone off from the night shift?

LETBY: Yes.

JOHNSON: So she would have gone, what, some time after 8 o'clock and probably before 9 o'clock; is that right?

LETBY: Yes.

JOHNSON: So [Father of Baby N]'s statement is correct that they came into the NNU as a result of a phone call?

LETBY: Yes, they did attend, yes.

JOHNSON: Do you remember him saying in his statement that -- sorry, [Father of Baby N]. Do you remember him saying in his statement that they went to get something to eat?

LETBY: Yes.

JOHNSON: Do you remember what he said happened when they went to get something to eat?

LETBY: That there was a deterioration with [Baby N].

JOHNSON: [Baby N] collapsed again.

LETBY: Yes.

JOHNSON: To be fair to you, he puts the time of that in the statement as earlier than the time of the collapse; okay?

LETBY: Right.

JOHNSON: But he says such was the dramatic and extreme nature of the collapse that they were offered a priest; do you remember that?

LETBY: Yes.

JOHNSON: And therefore that has to be, doesn't it, the collapse at 14.50?

LETBY: Yes, if that's the time of the collapse, yes.

JOHNSON: Well, let's look at tile 244, please.

LETBY: Yes.

JOHNSON: Your note?

LETBY: Yes.

JOHNSON: "Desaturation to 44. Heart rate below 100 at 90. Fresh blood..."

So more fresh blood?

LETBY: Yes.

JOHNSON: "... noted from mouth and 3ml aspirated from NG tube."

LETBY: Yes.

JOHNSON: So the source, on the face of it, not the stomach -- not the lungs, I should say, because the NG tube is in the stomach.

LETBY: Yes.

JOHNSON: What had you done to [Baby N] to make him bleed?

LETBY: I hadn't done anything to him.

JOHNSON: This is very similar to what happened to [Baby E], isn't it? Do you agree?

LETBY: Um, [Baby E] had significantly more blood, but yes, he did have fresh blood, yes.

JOHNSON: Do you remember telling Dr Mayberry about this?

LETBY: No.

JOHNSON: It's tile 245 in his note if we look at that, please.

If we scroll down, please, Mr Murphy, it's the entry made at -- and up a bit, sorry.

LETBY: I think that's Dr Saladi.

JOHNSON: That's Saladi, yes. Let's find Dr Mayberry. There he is at the bottom, Mr Murphy was right. This is written in retrospect of course:

"Crash bleeped to NNU due to sudden desaturation following 3ml aspirate of blood from NG."

LETBY: Yes.

JOHNSON: So we're talking about the same event, aren't we?

LETBY: Yes.

JOHNSON: "1 attempt at intubation by myself. Cords seen but obstructed by swelling in airway."

LETBY: Yes.

JOHNSON: Do you remember what he said about that?

LETBY: I remember him saying it was unusual, he'd not seen anything like that before.

JOHNSON: Exactly:

"Unlike anything I had encountered previously."

LETBY: Yes.

JOHNSON: You had shoved some foreign object down [Baby N]'s throat, hadn't you?

LETBY: Absolutely not, no.

JOHNSON: That's why he was bleeding.

LETBY: No.

JOHNSON: [Dr D] saw swelling in the throat.

LETBY: Yes.

JOHNSON: Dr Brearey saw blood in the throat about this time.

Right? Do you remember?

LETBY: Um, yes, if that's what he said, yes.

JOHNSON: And it's all your work, isn't it?

LETBY: No, it's not at all.

JOHNSON: Do you remember some assistance having to be called from Alder Hey?

LETBY: Yes.

JOHNSON: And you were agitated by that, weren't you?

LETBY: Yes, I'd never known anything like that.

JOHNSON: Mm.

LETBY: It was -- yeah.

JOHNSON: Did you think eyes from outside might work out what you'd been up to?

LETBY: No. There was nothing to do. I wanted them to come and help [Baby N] because at that point nobody on our unit could do that.

JOHNSON: Do you remember saying to [Dr D], "Who are these people"?

LETBY: Yes.

JOHNSON: "Who are these people?"

LETBY: Yes, I did. I had no idea who these people were. The nursery was full of people and I didn't know who they were. I'd never experienced staff coming from another department or another hospital before.

JOHNSON: So that's your reason, is it? You didn't know who they were?

LETBY: No, I didn't know who they were, no.

JOHNSON: You didn't know where they'd come from?

LETBY: I knew where they'd come from, I didn't know who they were or who was doing what. I have never known staff come from different hospitals or different departments.

It was a completely new experience.

JOHNSON: Do you remember the evidence from the doctors that as [Baby N] collapsed the final time, they were all away from him?

LETBY: As in every doctor?

JOHNSON: Yes. Dr Potter and Dr Lakin were in a different room, preparing Dr Potter's equipment, I think, and Dr Gibbs said they were in effect in a huddle, discussing what they were going to do.

LETBY: Okay.

JOHNSON: And that was the point at which [Baby N] collapsed again, wasn't it?

LETBY: Yes.

JOHNSON: And you took advantage of, as we have seen in many other cases, them being distracted to try to kill him?

LETBY: No.

JOHNSON: Because kill him -- you did try, didn't you?

LETBY: No.

JOHNSON: Were you telling the truth when you told the police that the only thing you could remember was what was written down about these events?

LETBY: Yes, at the start of the interview before I realised it was [Baby N], yes.

MR JUSTICE GOSS: We must break off, Mr Johnson.

MR JOHNSON: Yes.

MR JUSTICE GOSS: Will you be more than 1 minute finishing this?

MR JOHNSON: I think I've probably finished actually.

MR JUSTICE GOSS: Are you sure? We've got to finish at 3 and it is now --

MR JOHNSON: If I'm not sure, I'll cover the base in the morning, but if I take more time it won't be more than a couple of minutes.

MR JUSTICE GOSS: There we are. Thank you very much.

Right, ladies and gentlemen, 10.30 tomorrow morning then, please. Thank you very much. Remember your responsibilities as jurors: no communications, no research.

(3.01 pm)

(The court adjourned until 10.30 am on Thursday, 8 June 2023)